28

28 Physician

Physician's services, whether furnished in the office, the recipient's home, a hospital, a skilled nursing facility, or elsewhere, refer to services provided by a physician:

- Within the scope of practice of medicine or osteopathy as defined by state law; and
- By or under the personal supervision of an individual licensed under state law to practice medicine of osteopathy.

The policy provisions for physicians can be found in the *Alabama Medicaid Agency Administrative Code,* Chapter 6.

28.1 Enrollment

The Alabama Medicaid Agency's fiscal agent enrolls physicians and issues provider contracts to applicants who meet the licensure and/or certification requirements of the State of Alabama, the Code of Federal Regulations, the *Alabama Medicaid Agency Administrative Code*, and the *Alabama Medicaid Provider Manual*. For the purpose of enrollment, a physician is defined as: a physician who is fully licensed and possesses a current license to practice medicine.

The fiscal Agent also enrolls Physician Assistants (PA), Certified Registered Nurse Practitioners (CRNP), Certified Registered Nurse Anesthetists (CRNA), and Anesthesiology Assistants (AA) who are employed by a Medicaid enrolled physician or physician billing group. Physician-employed includes physicians practicing in an independent practice or in a group practice relationship. The collaborating physician is not required to be an actively enrolled Medicaid provider to participate in the Medicaid program, but the CRNP must be linked under an active billing group. The CRNP's collaborating physician must align with records from the Alabama Board of Medical Examiners (ALBME).

Refer to Chapter 38, Anesthesiology, for more information on CRNA and AA services.

Refer to Chapter 2, Becoming a Medicaid Provider, for general enrollment instructions and information. Failure to provide accurate and truthful information or intentional misrepresentation might result in action ranging from denial of application to permanent exclusion.

Federal requirements mandate providers re-validate periodically with the Alabama Medicaid program. Providers will be notified when they are scheduled to re-validate. Failure to re-validate and provide appropriate documentation to complete enrollment will result in an end-date being placed on the provider file. Once a provider file has been closed for failure to timely re-validate, providers will have to submit a new application for enrollment.

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National Provider Identifier, Type, and Specialty

A provider who contracts with Alabama Medicaid as a physician is added to the Medicaid system with the National Provider Identifiers provided at the time application is made. Appropriate provider specialty codes are assigned to enable the provider to submit requests and receive reimbursements for physician-related claims.

NOTE:
The 10-digit NPI is required when filing a claim.

Physicians are assigned a provider type of 31 (Physician). Nurse Practitioners are assigned a provider type of 09. Physician Assistants are assigned a provider type of 10. For a list of valid specialties for Nurse Practitioners and Physician Assistants, please refer to Chapter 21.

Valid specialties for physicians and practitioners include but are not limited to the list below:

Specialty (Physician - 31)	***Code***
Allergist	310
Anesthesiologist	311
Applied Behavior Analysis	175
Cardiac surgery	312
Cardiovascular disease	313
Cochlear implant team	740—See Chap. 10, Audiology/Hearing Services
Colon and rectal surgery	750
Dermatology	314
Dental Anesthesiology	279 (with inclusion of Specialty 311-Anesthsiology)
EENT	760
Emergency medicine Practitioner	315
Endocrinology	770
EPSDT	560
Family practice	316
Gastroenterology	317
General practice	318
General surgery	319
Geriatrics	320
Hand surgery	321
Hematology	780
Infectious diseases	790
Internal medicine	800
Mammography	292
Mobile Provider	299
Neonatology	323
Nephrology	324
Neurological surgery	325
Neurology	326
Nuclear medicine	327
Nutrition	230
Obstetrics/Gynecology	328
Oncology	329

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Specialty (Physician - 31)	*Code*
Ophthalmology	330
Oral and maxillofacial surgery	272
Orthopedic	810
Orthopedic surgery	331
Otorhinolaryngology	332
Pathology	333
Pediatrics	345
Plastic, reconstructive, cosmetic surgery	337
Primary care provider (not a screening provider but can refer patients)	720
Proctologist	338
Psychiatrist	339
Pulmonary disease Specialist	340
Radiology	341
Rheumatology	830
Telemedicine	931
Thoracic surgeon	342
Urologist	343
Vascular surgery	313

Enrollment Policy for Physicians

Providers (in-state and out-of-state) who meet enrollment requirements are eligible to participate in the Alabama Medicaid Program. The PA or CRNP must send a copy of the prescriptive authority granted by the licensing board in order for the PA or CRNP to be added to the Provider License File for the purpose of reimbursing the pharmacist for the prescriptions written by the PA or CRNP. This copy must be sent to Gainwell Provider Enrollment, P.O. Box 241685, AL 36124-1685.

Gainwell will not enroll physicians having limited licenses unless complete information as to the limitations and reasons are submitted in writing to the Provider Enrollment Unit for review and consideration for enrollment.

Enrollment Policy for Primary Care Enhanced Physician Rates or BUMP Rate

In order for PCPs to qualify for BUMP rates, the following requirements must be met:

A. A provider must meet one of the following requirements listed below:

1. A provider is eligible1 who has a specialty or subspecialty designation in family medicine, general internal medicine, or pediatrics that is recognized by the American Board of Medical Specialties (ABMS), the American Board of Physician Specialties (ABPS), or the American Osteopathic Association (AOA), and practice in their specialty.

2. A NON-board certified provider who practices in the field of family medicine, general internal medicine, or pediatrics or a subspecialty under one of these specialties, is eligible1 if he/she can attest that 60 percent of their paid Medicaid procedures billed are for certain specified procedure codes for evaluation and management (E&M) services and certain Vaccines for Children (VFC) vaccine administration codes.

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B. The following providers do not meet criteria for BUMP rates:

1. Federally Qualified Health Center (FQHC)
2. Rural Health Clinic (RHC)
3. Health Department (ADPH)
4. Teaching Facility
5. OB/GYNs
6. Nursing Facility

C. Eligible providers may enroll by:

1. Visit www.medicaid.alabama.gov
2. Select Provider Enrollment from the drop-down menu under “Providers” and then “Forms for Provider Enrollment and Revalidation”
3. Complete the “Bump” Self-Attestation Form
4. Attach proof of board certification, if applicable
5. Mail original self-attestation form to the address specified

D. Providers will receive payment at the enhanced rates for dates of service beginning with the date the self-attestation is entered into the system by the fiscal agent.

[1]As defined by 42 C.F.R. Pts.438, 441, and 447, interpreted by Centers for Medicare and Medicaid (CMS) guidance and consistent with the Alabama State Plan.

Providers that meet the following criteria are eligible to participate in the Physician Teaching Facility (PTF) program or Physician Retention and Access Improvement Program (PRAIP).

Physician Teaching Facilities [PTF]

In order to maintain adequate access to specialty faculty physician (all specialties including general practice, family practice, and general pediatrics) services as required, supplemental payments will be made for services provided to Medicaid recipients by eligible physicians and other professional services practitioners.

To qualify for the supplemental payments, eligible physicians and other professional service practitioners
must:
1. Be enrolled as one of the following provider types:

a. Physicians (as defined in state plan)
b. Physician Assistants
c. Nurse Practitioners (NPs)
d. Certified Nurse Midwives (CNM)
e. Certified Registered Nurse Anesthetists (CRNAs) or Anesthesiology Assistants (AAs)
f. Clinical Psychologists

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g. Optometrists

2. Be in a hospital sponsored location as an approved place of service:
   a. Inpatient hospital
   b. Outpatient hospital
   c. Hospital-based clinic
   d. Hospital affiliated clinic
3. Be licensed by the State of Alabama, have an Alabama Medicaid provider agreement and be employed by or under contract with a medical school that is part of the public university system or a children's hospital healthcare system which meets the criteria and receives funding under Section 340E (a) of the U.S. Public Health Services Act (42 U.S.C. 256e) and which operates and maintains a state license for specialty pediatric beds. Participants that currently qualify are:
   a. The University of Alabama System
   b. The University of South Alabama
   c. Children's of Alabama

The services listed below do not qualify under the Physician Access (Enhanced) Payments Teaching Physicians:

a) Clinical diagnostic lab procedures
b) Technical component of radiology services
c) Services provided to dual eligibles
d) EPSDT
e) Injectables

Non-state governmental entity (Specifically a health care authority or a wholly owned subsidiary thereof

Physician Retention and Access Improvement Program [PRAIP]

In order to maintain adequate access to specialty physicians (all specialties, including general practice, family practice, and general pediatrics) services as required, supplemental payments will be made for services provided to Medicaid recipients by eligible physicians and other professional services practitioners.

To qualify for the supplemental payments, eligible physicians and other professional service practitioners must:

1. Be one of the following provider types:
   a. Physicians (as defined in state plan)
   b. Physician Assistants
   c. Nurse Practitioners (NPs)
   d. Certified Nurse Wives (CNMs)
   e. Certified Registered Nurse Anesthetists (CRNAs) or Anesthesiology Assistants (AAs)
   f. Clinical Psychologists
   g. Optometrists
2. Be in a hospital-sponsored location as an approved place of service:
   a. Inpatient hospital
   b. Outpatient hospital
   c. Hospital-based clinic
   d. Hospital affiliated clinic

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3. Be licensed by the State of Alabama, have an Alabama Medicaid provider agreement and be employed
by or under contract with a non-state governmental entity, specifically a health care authority or a
wholly owned subsidiary thereof; and be a participant in the PRAIP. Participants that currently qualify are:

a. The Health Care Authority for Baptist Health an Affiliate of UAB Health System
b. The Health Group of Alabama, LLC, a wholly owned subsidiary of The Health Care
Authority of The City of Huntsville, Alabama
c. Houston County Health Care Authority D/B/A as Southeast Alabama Medical Center
d. The East Alabama Healthcare Authority
e. The Health Care Authority of the City of Anniston dba Northeast Alabama Regional Medical Center
f. The Sylacauga Healthcare Authority

Added: The East Alabama…Sylacauga Healthcare Authority

The services listed below do not qualify under PRAIP:

a. Clinical diagnostic lab procedures
b. Technical component of radiology services
c. Services provided to dual eligibles
d. EPSDT
e. Injectables

For supplemental payment formula, see State Plan Attachment 4.19B.

28.2 Benefits and Limitations

This section describes program-specific benefits and limitations. Refer to Chapter 3, Verifying Recipient Eligibility, for general benefit information and limitations. Refer to Chapter 7, Section 7.1.1 - “Medical Necessity/Medically Necessary Care”, for general criteria on Medical Necessity/ Medically Necessary Care.

Physicians are expected to render medically necessary services to Medicaid patients in the same manner and under the same standards as for their private patients. Physicians are to bill the Alabama Medicaid Agency their usual and customary fee.

Office visits are limited to one per day, per recipient, per provider. For purposes of this limitation, a physician or physician of the same specialty and subspecialty from the same group practice are considered a single provider. Annual office visit benefit limits are 14 office visits per calendar year.

Medicaid will no longer require physicians enrolled in and providing services through a **residency** training program be assigned a pseudo Medicaid license number to be used on prescriptions written for Medicaid recipients. Effective for claims submitted on or after January 1, 2012, interns and non-licensed residents must use the NPI or license number of the teaching, admitting, or supervising physician.

Written medication prescriptions should have a typed or printed name of the prescriber on the prescription and the handwriting must be legible.

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Pharmacists **must have the physician's license number** prior to billing for prescriptions. Pharmacies shall use the correct physician license number when submitting a pharmacy claim to Medicaid.

Supervising physicians may bill for services rendered to Medicaid recipients by residents enrolled in and providing services through (as part of) an approved residency training program. The following rules shall apply to physicians supervising residents as part of an approved residency training program:

a. The supervising physician shall sign and date the admission history and physical progress notes written by the resident.

b. The supervising physician shall review all treatment plans and medication orders written by the resident.

c. The supervising physician shall be available by phone or pager.

d. The supervising physician shall designate another physician to supervise the resident in his/her absence.

e. The supervising physician shall not delegate a task to the resident when regulations specify that the physician perform it personally or when such delegation is prohibited by state law or the facility's policy.

Payments from Medicaid funds can be made only to physicians who provide the services; therefore, no reimbursement can be made to patients who may personally pay for the service rendered.

The physician agrees when billing Medicaid for a service that the physician will accept as payment in full, the amount paid by Medicaid for that service, plus any cost-sharing amount to be paid by the recipient, and that no additional charge will be made. Conditional collections from patients, made before Medicaid pays, which are to be refunded after Medicaid pays, are not permissible. The physician may bill the patient, in addition to the cost-sharing fee, for services rendered when benefit limitations are exhausted for the year or when the service is a Medicaid non-covered benefit. However, the provider (or their staff) must advise each patient prior to services being rendered when Medicaid payment will not be accepted, and the patient will be responsible for the bill. If a provider routinely accepts a Medicaid assignment, he or she may not bill Medicaid or the recipient for a service he or she did not provide, i.e., missed or canceled appointment.

A hospital-based physician who is a physician employed by and paid by a hospital may not bill Medicaid for services performed therein and for which the hospital is reimbursed. A hospital-based physician shall bill the Medicaid Program on a CMS-1500, Health Insurance Claim Form or assign their billing rights to the hospital, which shall bill the Medicaid Program on a CMS-1500 form. A hospital-based physician who is not a physician employed by and paid by a hospital shall bill Medicaid using a CMS-1500 claim form.

A physician enrolled in a residency training program and whose practice is limited to the institution in which that resident is placed shall not bill Medicaid for services performed therein for which the institution is reimbursed through the hospitals' cost reports. For tracking purposes, these physicians will be assigned pseudo Medicaid license numbers.

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Hospital-based physicians are reimbursed under the same general system as is used in Medicare. Bills for services rendered are submitted as follows:

- All hospital-based physicians, including emergency room physicians, radiologists, and pathologists, will bill Medicaid on a CMS-1500 claim form, or assign their billing rights to the hospital, which shall bill Medicaid on a CMS-1500 claim form.
- Physician services personally rendered for individual patients will be paid only on a reasonable charge basis (i.e., claims submitted under an individual NPI on a physician claim form). This includes services provided by a radiologist and/or pathologist.
- Reasonable charge services are: 1) personally furnished for a patient by a physician; 2) ordinarily require performance by a physician and; 3) contribute to the diagnosis or treatment of an individual patient.

Out-of-State Services

Alabama Medicaid will pay for services furnished in another state to the same extent it would pay for services furnished within its boundaries as long as the service is provided to an Alabama Medicaid recipient and any of four following conditions is met:

(a) Medical services are needed due to a medical emergency.
(b) Medical services are needed, and the recipient's health would be endangered if he were required to travel to his state of residence.
(c) The state determines, on the basis of medical advice, that the needed medical services, or necessary supplementary services, are more readily available in the other state.
(d) It is general practice for recipients in a particular locality to use medical resources in another state.

Out-of-state providers must follow the enrollment procedures of the Alabama Medicaid Agency. Please refer to Chapter 2 - Becoming a Medicaid Provider, for detailed information. All program policies apply regardless of where care is provided.

Medical care and services that require prior authorization for in-state providers will continue to require prior authorization for out-of-state providers.

The following is required prior to submitting an out-of-state service request:

- The requested services are not available in-state, and
- The out-of-state provider is enrolled or willing to enroll as an Alabama Medicaid Provider, and
- The out-of-state provider is agreeable to Alabama Medicaid's per diem rate and provider fee schedules.

To request an out-of-state service, complete and submit an *Out-of-State Service Request* Form 382 by following the steps below:

(a) Locate the Out-of-State Service Request Form 382 under the "Programs" tab on Alabama Medicaid's website. Refer to instructions

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382-A for further guidance. Form 382 is to be completed by the in-state referring provider.

(b) Obtain the out-of-state provider's Alabama Medicaid ID number. See Chapter 2 – Becoming a Medicaid Provider for further information.

(c) Attach the following medical documentation outlined in part IX of Form 382:
- Letter of Medical Necessity supporting the need for out-of-state service along with documentation ensuring all local and state resources have been exhausted.
- Consult note from University of Alabama at Birmingham, University of South Alabama, or Children's of Alabama detailing evaluation and professional treatment plan recommendation.
- Any relevant medical, surgical, and/or consultation notes, lab/radiology results, pathology reports, and/or recent hospital History and Physical (H&P) and discharge summary
- Detailed care plan from the out-of-state provider.

(d) Please ensure that all sections of Form 382 are filled in completely and accurately. Incomplete submissions will be returned to the requestor for completion.

Submit Form 382 along with all supporting documentation via encrypted email to: out-of-state-request@medicaid.alabama.gov The program manager will review your submission and communicate with you regarding next steps.

Off-site Mobile Physician Services

Off Site Mobile Physician's Services shall comply with all Medicaid rules and regulations as set forth in the State Plan, Alabama Medicaid Administrative Code, and Code of Federal Regulations including but not limited to the following requirements:

(a) Shall provide ongoing, follow-up, and treatment and/or care for identified conditions,

(b) Shall provide ongoing access to care and services through the maintenance of a geographically accessible office with regular operating business hours within the practicing county or within 15 miles of the county in which the service was rendered,

(c) Shall provide continuity and coordination of care for Medicaid recipients through reporting and communication with the Primary Medical Provider,

(d) Shall maintain a collaborative effort between the off-site mobile physician and local physicians and community resources. A matrix of responsibility shall be developed between the parties and available upon enrollment as an off-site mobile physician,

(e) Shall provide for attainable provider and recipient medical record retrieval,

(f) Shall maintain written agreements for referrals, coordinate needed services, obtain prior authorizations and necessary written referrals for services prescribed. All medical conditions identified shall be referred and coordinated, for example:

(i) Eyeglasses,

(ii) Comprehensive Audiological services,

(iii) Comprehensive Ophthalmological services,

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(g) Shall not bill Medicaid for services which are offered to anyone for free. Provider shall utilize a Medicaid approved sliding fee scale based on Federal Poverty Guidelines,

(h) Shall ensure that medical record documentation supports the billing of Medicaid services, and

(i) Shall obtain signed and informed consent prior to treatment.

NOTE:
If a provider routinely accepts Medicaid assignments, he/she may not bill Medicaid or the recipient for a service he/she did not provide, i.e., "no call" or “no show”.

Locum Tenens and Substitute Physician Under Reciprocal Billing Arrangements

It is common practice for physicians to retain substitute physicians to take over their professional practices when the regular physicians are absent for reasons such as illness, pregnancy, vacation, or continuing medical education, and for the regular physician to bill and receive payment for the substitute physician’s services as though he/she performed them. The substitute physician generally has no practice of his/her own and moves from area to area as needed. The regular physician generally pays the substitute physician a fixed amount per diem, with the substitute physician having the status of an independent contractor rather than of an employee. The substitute physicians are generally called “locum tenens” physicians.

Reimbursement may be made to a physician submitting a claim for services furnished by another physician in the event there is a reciprocal arrangement. The regular physician shall identify the services as substitute physician services by entering HCPCS modifier **Q5** (Service Furnished by a Substitute Physician under a Reciprocal Arrangement) or HCPCS modifier **Q6** (Service Furnished by a Locum Tenens Physician) after the procedure code. The reciprocal arrangement may not exceed 14 days in the case of an informal arrangement. Effective for claims submitted on or after June 15, 2012, the reciprocal arrangement may not exceed 60 continuous days in the case of an arrangement involving per diem or other fee-for-time compensation. Both providers participating in a reciprocal arrangement must be enrolled with the Alabama Medicaid Agency. The regular physician should keep a record on file of each service provided by the substitute physician and make this record available to Medicaid upon request. Claims will be subject to post-payment review. Please refer to section 28.6.3 Procedure Codes and Modifiers for information regarding modifiers Q5 and Q6.

Pharmacy Quantity Limitations and Controlled Substances

The pharmacist or prescriber must request an override when the prescription exceeds Medicaid’s maximum limit allowed per month. The prescriber should not write separate prescriptions, one to be paid by Medicaid and one to be paid as cash, to circumvent the override process. For further information on pharmacy quantity limitations and prescriptions for controlled substances, refer to Chapter 27, section 27.2.3 “Quantity Limitations”.

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NOTE:
In accordance with Section 5042 of the SUPPORT Act, effective October 1, 2021, prescribers of Medicaid eligible recipients are required to check the Alabama PDMP (Prescription Drug Monitoring Program) prior to prescribing a Schedule II controlled substance. If the prescriber does not check the PDMP, the prescriber is required to document the reason in the medical record. Exclusions to this requirement include prescriptions written for hospice patients, patients with an active cancer diagnosis, residents of a long term care nursing facility, and children under the age of 18(Schedule II prescriptions for ADHD only).

28.2.1 Physician-Employed Practitioner Services

Medicaid payment may be made for the professional services of the following physician-employed practitioners:

- Physician Assistants (PAs)
- Certified Registered Nurse Practitioners (CRNPs)

Nurse Practitioner is defined as a Registered Professional Nurse who is currently licensed to practice in the state, who meets the applicable State of Alabama requirements governing the qualifications of nurse practitioners.

Physician Assistant is a person who is a graduate of an approved program, is licensed by the Board of Medical Examiners (BME) of the State of Alabama, and is registered by the BME to perform medical services under the supervision of a physician approved by the BME to supervise the assistant.

All services requiring additional education and training beyond the scope of practice billed by a CRNP/PA must be documented in the approved collaborative agreement from the BME and the Alabama Board of Nursing (ABN) between the practitioner and physician. The only exception is for those "routine" services within the scope of practice approved by the applicable licensing and governing boards. Services billed outside a CRNP/PA scope of practice and/or collaborative agreement are subject to post-payment review.

Medicaid may make payment for services of PAs and CRNPs who are legally authorized to furnish services and who render the services under the supervision of an employing physician with payment made to the employing physician. Medicaid will not make payment to the PA or CRNP. Generally, CRNPs and PAs are reimbursed at 80% of the allowed amount for all services except lab and injectables, which should pay at 100%.

The employing physician must be a Medicaid provider in active status. The employing physician need not be physically present with the PA or CRNP when the services are being rendered to the recipient; however, the physician must be immediately available to the PA or CRNP for direct communication by radio, telephone, or telecommunication. The PA's or CRNP's employing physician is responsible for the professional activities of the PA or CRNP and for assuring that the services provided are medically necessary and appropriate for the patient. There shall be no unsupervised practice by PAs or CRNPs.

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The PA or CRNP **must enroll with the Alabama Medicaid Agency** with a valid NPI number and employing physician as the payee.

Covered services furnished by the PA or CRNP must be billed under the PA's or CRNP's name and NPI as the rendering provider.

The office visits performed by the PA or CRNP will count against the recipient's yearly benefit limitation.

The PA or CRNP may make physician-required visits to nursing facilities.

The PA or CRNP may not make physician-required inpatient visits to hospitals or other institutional settings to qualify for payment to the physician or to satisfy current regulations as physician visits.

Effective August 01, 2018, an initial prescription or order for home health services and certain medical supplies, equipment and appliances must be signed by a physician.

Refer to Chapter 14 – Durable Medical Equipment (DME), supplies, Appliances, Prosthetics, Orthotics and Pedorthics (POP) for more information on the requirements for the initial written prescription/order for certain medical supplies, equipment, and appliances.

Refer to Chapter 17 – Home Health for more information on the requirements for placing the initial written prescription/order for home health services.

Refer to Chapter 21 – Certified Registered Nurse Practitioner (CRNP) and Physician Assistant (PA) for additional information. Information regarding Independent CRNPs is also found in this chapter. For more specific information on coverage, you may call the Provider Assistance Center at 1-800-688-7989.

28.2.2 Covered Services

In general, Medicaid covers physician services if the services meet the following conditions:

- Considered medically necessary by the attending physician
- Designated by procedure codes in the Physicians' Current Procedural Terminology (CPT), or HCPCS. The table below contains details on selected covered services.

Consistent with the implementation of the mandated Medicaid NCCI edits effective November 9, 2010. Refer to this link, https://www.medicaid.gov/medicaid/program-integrity/national-correct-coding-initiative-medicaid/index.html for more information regarding NCCI.

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Abdominoplasty	Medical necessity criteria must be met and Prior Authorization obtained, or the procedure will be considered cosmetic and will not be covered. See Chapter 4, Obtaining Prior Authorization.
Add-on Code	Add-on Code definition in the CPT is recognized and allowed for payment with the appropriate primary code.
Administration Fee	Please refer to Appendix H, Medicaid Physician Administered Drugs, section H.1 (Policy) for information regarding office visits, chemotherapy, and administration fees. When an Evaluation and Management service is provided *and* a Drug Administration code (96372, 96373, 96374, 96375, and 96376) is provided at the same time, the E&M code, Drug Administration Code, and the HCPCS Code for the drug may be billed. A **Significant Separately Identifiable Service** must be performed in conjunction with the Drug Administration code for consideration of payment for the E&M Code. A **Modifier 25** must be appended to the E&M service for recognition as a "**Significant Separately Identifiable Service".** Medical Record documentation must support the medical necessity of the visit as well as the level of care provided. However, when no **Significant Separately Identifiable** E&M service is actually provided at the time of a Drug Administration, an E&M code should not be billed. In this instance, the Drug Administration Code and the HCPCS Code for the drug may be billed. An example of this is routine monthly injections like B-12, iron, or Depo-Provera given on a regular basis without a **Significant Separately Identifiable** E&M service being provided. These services will be subject to post payment review. Please refer to Appendix A, Well Child Check-Up, Section A.6.1 (Fees) for information regarding the use of designated VFC codes for billing immunization administration fee(s).
Allergy Treatments	Please refer to Appendix H, Medicaid Physician Administered Drugs for information.
Applied Behavior Analysis (ABA)Therapy	ABA therapy is covered. See Chapter 37 – Therapy.
Anesthesia	Anesthesia is covered. See Chapter 38, Anesthesiology.
Artificial Eyes	Artificial eyes must be prescribed by a physician. Refer to Chapter 15, Eye Care Services for specific coverage information.
Bariatric Procedures	Considered cosmetic unless specific medical criteria are met and with Prior Authorization. Bariatric surgical procedures are considered for Medicaid eligible recipients between 18 and 64 years of age, effective June 1, 2009. Bariatric surgery for recipients who are under 18 years old and who have one or more immediate life-threatening co-morbidities will be considered for authorization on a case-by-case basis by the Medical Director, effective March 1, 2014. See Chapter 4, Obtaining Prior Authorization.
Breathing or Inhalation Treatments	Breathing or inhalation treatments are a covered service. Any medication provided during a breathing treatment (e.g., Albuterol) is considered a component of the treatment charge. EXCEPTION: See Appendix H related to coverage of J2545 Pentamidine Isethionate.
Cardiac Catheterization	Cardiac Catheterization codes may be subject to the multiple procedure/surgery reductions.

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Cerumen Removal	CPT code 69210 is a covered service. **Payment may be made for impacted cerumen** (when **ALL** of the following are met): 1) the service is the sole reason for the patient encounter, 2) the service is personally performed by the physician or non-physician practitioner (i.e. nurse practitioner, physician assistant), 3) the service is provided to a patient who is symptomatic, and 4) the documentation illustrates significant time and effort spent in performing the service. Effective January 1, 2014, CPT code 69210 is a unilateral procedure. Please refer to section 28.6.3 for billing of bilateral procedures. **Payment consideration may be made for both the procedure and the E&M services** if **ALL** of the following conditions exist: 1) The nature of the E&M visit is for something other than removal of impacted cerumen. 2) During an unrelated patient encounter (visit), a specific complaint or condition related to the ear(s) is either discovered by or brought to the attention of the physician/non-physician practitioner by the patient. 3) Otoscopic examination of the tympanic membrane is not possible due to a cerumen obstruction in the canal. 4) The removal of impacted cerumen requires the expertise of a physician or non-physician practitioner. 5) The procedure requires a significant amount of the physician'/non-physician practitioner's effort and time. 6) Documentation is present in the patient record to identify the above criteria have been met. **Limitations**: • Removal of impacted cerumen performed by someone other than the physician or non-physician practitioner is not billable. • Simple cerumen removal performed by the physician or office personnel (e.g., nurses, office technicians) is not medically necessary and therefore, not separately payable. • An E&M service and the removal of impacted cerumen are not separately payable when the sole reason for patient encounter is for the removal of impacted cerumen. • The patient is asymptomatic (e.g. denies pain, hearing loss, vertigo. etc.). • Visualization aids such as, but not necessarily limited to, binocular microscopy, are considered to be included in the reimbursement for 69210 and should not be billed separately. Most patients do not require medically necessary disimpaction of cerumen by a physician. Patients who require this service more often than 3-4 times per year would be unusual. **NOTE: CPT 69210 requires a prior authorization and is restricted to ages 0-20 in the outpatient hospital setting.
Chemotherapy Administration	When an Evaluation and Management service is provided *and* a Hydration, Therapeutic, Prophylactic, Diagnostic and Chemotherapy Administration code is provided at the same time, the E&M code, Drug Administration Code, and the HCPCS Code for the drug may be billed. A **Significant Separately Identifiable Service** must be performed in conjunction with these administration codes for consideration of payment for the Evaluation and Management Code. A **Modifier 25** must be appended to the E & M service for recognition as a **"Significant Separately Identifiable Service".** Procedure Code 99211 will not be allowed with a modifier 25 or when billed in conjunction with the above administration codes. Medical record documentation must support the medical necessity and level of care of the visit. These services are subject to post payment review.

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CT Scans	CT scans are covered as medically necessary. Effective for dates of service March 2, 2009, and thereafter, CT scans require prior authorization for coverage. See Chapter 4, Obtaining Prior Authorization, Chapter 19, Hospital and Chapter 22, Independent Radiology.
Chiropractors	Chiropractic services are covered only for QMB recipients and for services referred directly as a result of an EPSDT screening. Chiropractic services are available up to 30 days prior to release for an eligible child under age 21 being held "post adjudication" as an inmate of a public institution. (Adjudication is the court process that determines if an individual committed the act for which they were charged).
Chromosomal Studies	Refer to Chapter 24: Maternity Care for further information.
Circumcision	Circumcision of newborns is a covered service. If medically necessary, non-newborn circumcision is covered. These services will be subject to post payment review.
Dental Varnishing	Refer to Chapter 13, Dentist for specific coverage information.
Developmental Testing Intensive Level (Multi-disciplinary Team only)	Refer to Appendix A, Well Child Check-Up (EPSDT) for specific coverage information.
Diet Instruction	Diet instruction performed by a physician is considered part of a routine visit.
Drugs	Refer to Appendix H, Physician Drug List for coverage information.
Endovenous **Laser** Ablation of Varicose Veins, Endoluminal **Radiofrequency** Ablation of Saphenous Varicose Veins, Sclerotherapy, and Ambulatory phlebectomy	The following procedure codes require prior authorization before services are rendered to a recipient: 1. Procedure codes**36478** (Endovenous ablation therapy of incompetent vein, extremity, inclusive of all imaging guidance and monitoring, percutaneous, **laser**; first vein treated) and **Add on** code **36479** (second and subsequent veins treated in a single extremity, each through separate access sites) should only be billed along with the primary code (36478). 2. Procedure codes **36475** (Endovenous ablation therapy of incompetent vein, extremity, inclusive of all imaging guidance and monitoring, percutaneous, **radiofrequency**; first vein treated, and **Add on** code **36476** (second and subsequent veins treated in a single extremity, each through separate access sites) should only be billed along with the primary code (36475). 3. **Effective 1/1/2013**, the following procedure codes will require prior authorization: **36470** (Injection of **sclerosing** solution; single vein), **36471** (multiple veins, same leg), **37765** (Stab **phlebectomy** of varicose veins, 1 extremity; 10-20 stab incisions), and **37766** (more than 20 incisions). ** **These procedures are not covered for cosmetic purposes.**
Eustachian Tube Inflation	Effective 8/25/2008, only physicians with specialties of EENT and Otorhinolaryngology may bill eustachian tube inflation, transnasal; with catheterization (69400), without catheterization (69401).

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Examinations	Physician visits for examinations are counted as part of each recipient's benefit limit of 14 physician visits per year. Exception: Certified Emergencies. Office visits are limited to one per day, per recipient, per provider. For purposes of this limitation, a physician or physician of the same specialty and subspecialty from the same group practice are considered a single provider. Annual routine physical examinations are not covered except through EPSDT. Refer to Appendix A, EPSDT, for details. Medical examinations for such reasons as insurance policy qualifications are not covered. Physical examinations for establishment of total and permanent disability status if considered medically necessary are covered. Medicaid requires a physician's visit once every 60 days for patients in a nursing facility. Patients in intermediate care facilities for the individuals with intellectual disabilities must receive a complete physical examination at least annually.
Eyecare	Eye examinations by physicians are a Medicaid covered service. Physician visits for eye care disease are counted as part of each recipient's benefit limit of 14 physician visits per year.
Foot Devices	See Chapter 14, Durable Medical Equipment (DME), for details
Gastric bypass	Covered with prior authorization approval when specific medical criteria are met. See Chapter 4, Obtaining Prior Authorization.
Hearing Aids	See Chapter 10, Audiology/Hearing Services, for details.
Hyperbaric Oxygen Therapy	Topically applied oxygen is not hyperbaric and is not covered. HBO therapy should not be a replacement for other standard successful therapeutic measure. Medical necessity for the use of hyperbaric oxygen for more than two months must be prior approved. (Chapter 4, Obtaining Prior Authorization). Refer to Chapter 19 Hospital, under Outpatient Hyperbaric Oxygen Therapy (HBO) for specific coverage information.
Hyperalimention Parental TPN IDPN IPN	Please refer to Section 28.2.9 for documentation requirements for parental, TPN, IDPN, and IPN nutrition.
Immunizations	Refer to Appendix A, EPSDT, for information regarding the Vaccines For Children (VFC) Program. Payment for immunizations against communicable diseases for adults will be made if the physician normally charges his patients for this service. Refer to Appendix H, Alabama Medicaid Physician Administered Drugs for coverage information.
Infant Resuscitation	Newborn resuscitation (procedure code 99465 on/after 01/01/09) is a covered service when the baby's condition is life threatening and immediate resuscitation is necessary to restore and maintain life functions. Intubation, endotracheal, emergency procedure (procedure code 31500) cannot be billed in conjunction with newborn resuscitation.
Long Acting Reversible Contraception (LARC)	Effective for dates of service April 1, 2014, and thereafter, Alabama Medicaid will cover long acting birth control in the inpatient hospital setting after a delivery for postpartum women or in an outpatient setting after discharge from the inpatient hospital. Refer to Chapter 19, Hospital for additional information.
Mammography Diagnostic	Refer to Chapter 22, Independent Radiology for coverage information.
Mammography Screening	Refer to Chapter 22: Independent Radiology for coverage information.
Medical Materials and Supplies	Costs for medical materials and supplies normally utilized during office visits or surgical procedures are to be considered part of the total fee for procedures performed by the physician and therefore are not generally a separately billable service.

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Medical Necessity	The Alabama Medicaid Agency is mandated to only reimburse for services, procedures, and surgeries that are medically necessary. Refer to Chapter 7, Understanding Your Rights and Responsibilities as a Provider for general criteria on Medical Necessity/ Medically Necessary Care. Medical necessity must be clearly documented in the recipient's medical record with supporting documentation such as: Laboratory test results, diagnostic test results, history (past attempts of management if applicable), signs and symptoms, etc. All Medicaid services are subject to retrospective review for medical necessity. EXAMPLE: Endometrial Ablation is covered by Medicaid when it is considered medically necessary and should not be performed when an alternative outcome is intended such as cessation of menses.
Nerve Conduction Studies and Electromyography	Refer to Chapter 22, Independent Radiology for coverage information.
Newborn Claims	Five kinds of newborn care performed by physicians in the days after the child's birth when the mother is still in the hospital may be filed under the mother's name and number or the baby's name and number: When billing under the mother's Medicaid number, use diagnosis codes **Z76.1-Z76.2 and Z00.129 for ICD-10, for normal newborn care. These diagnosis codes must be used on the claim form for consideration of payment.** 1. Routine newborn care (99460 on/after 01/01/09, 99462 on/after 01/01/09, and discharge codes 99238 or 99239). 2. Circumcision (54150 or 54160) Please note that the billing of PC 64450 is not allowed along with PC 54150 (which includes the nerve block in the description). 3. Newborn resuscitation (99465). 4. Standby services following a cesarean section or a high-risk vaginal delivery (99360). 5. Attendance at delivery (when requested by delivering physician) and initial stabilization of newborn (99464). Standby services (procedure code 99360) are covered only when the pediatrician, family practitioner, neonatologist, general practitioner, or non-delivering OB/GYN is on standby in the operating or delivery room during a cesarean section or a high-risk vaginal delivery. Attendance of the standby physician in the hospital operating or delivery room must be documented in the operating or delivery report. Use CPT codes when filing claims for these five kinds of care. If these services are billed under the mother's name and number and the infant(s) are twins, indicate Twin A or Twin B in Block 19 of the claim form. Any care other than routine newborn care for a well-baby, before and after the mother leaves the hospital, must be billed under the child's name and number.

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Newborn Hearing Screening	Inpatient newborn hearing screenings are considered an integral part of inpatient hospital services. Please refer to Chapter 19 for Outpatient Services. Limited hearing screen codes 92586 and 92587 (CPT 2002) may be billed in an outpatient setting provided: 1) an infant was discharged prior to receiving the inpatient hearing screen, or 2) an infant was born outside a hospital or birthing center. These codes are reimbursable for audiologists, pediatricians, otolaryngologists, and EENT. Comprehensive hearing screen codes 92585/92588/92558 may be billed for: 1) infants who fail the newborn hearing screening prior to discharge from the hospital, or 2) infants/children fail a hearing screening at any time following discharge. Comprehensive hearing screenings should be performed on infants by three months of age if they failed the newborn hearing screening prior to discharge. Code 92585 is reimbursable for otolaryngologists, audiologists, pediatricians, and EENT. Code 92588 is reimbursable for otolaryngologists, audiologists, pediatricians, EENT, and neurologists.
Obstetrical Services	Refer to Chapter 24: Maternity Care for further information.
Obstetrical Ultrasounds	Refer to Chapter 24: Maternity Care for further information.
Oxygen and Compressed Gas	A physician's fee for administering oxygen or other compressed gas is a covered service under the Medicaid program. Oxygen therapy is a covered service based on medical necessity and requires prior authorization. Please refer to Chapter 14, DME, for more information.
Podiatrist Service	Covered for QMB or EPSDT referred services only. See Chapter 29, Podiatrist, for more details. Podiatrist services are available up to 30 days prior to release for an eligible child under age 21 being held "post adjudication" as an inmate of a public institution. (Adjudication is the court process that determines if an individual committed the act for which they were charged).
Post-Surgical Visits	Routine post-surgical care in the hospital or office visits for conditions directly related to major surgical procedures are covered by the surgical fee. Post-surgical visits cannot be billed separately the day of, or up to 90 days after surgery. For conditions unrelated to the surgical procedure bill the appropriate (E&M) procedure code with a 24 modifier appended. The diagnosis must support use of the modifier 24.
Prosthetic Devices	Internal prosthetic devices (e.g., Smith Peterson Nail or pacemaker) are a covered benefit.
Psychiatric Services	Physician visits for psychiatric services are counted as part of each recipient's benefit limit of 14 physician visits per year. Psychiatric evaluation or testing are covered services under the Physicians' Program if services are rendered by a physician in person and are medically necessary. Psychiatric evaluations are limited to one per calendar year, per provider, per recipient. Psychotherapy visits are included in the office visit limit of 14 visits per calendar year. Effective January 1, 2013, the following psychotherapy Add-on codes may be billed in conjunction with Evaluation and Management codes billed by the psychiatrist: 90833, 90836, and 90838. Psychiatric services under the Physicians' Program are confined to use with psychiatric diagnosis (F0150-F99 for ICD-10) and must be performed by a physician. The "must be performed by a physician" does not apply for EPSDT-referred psychiatric services. Hospital visits are not covered when billed in conjunction with psychiatric therapy on the same day. For EPSDT-referred services rendered by psychologist, see Chapter 34 for details. Psychiatric day care is not a covered benefit under the Physicians' Program. **NOTE**: For billing purposes, psychiatrist services are not limited to what psychologist bills.

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Radiation Treatment Management	Radiation treatment management services do not need to be furnished on consecutive days. Up to two units may be billed on the same date of service as long as there has been a separate break in therapy sessions.
Second Opinions	Physician visits for second opinions are counted as part of each recipient's benefit limit of 14 physician visits per year. Optional Surgery: Second opinions (regarding non-emergency surgery) are highly recommended in the Medicaid program when the recipients request them. Diagnostic Services: Payment may be made for covered diagnostic services deemed necessary by the second physician.
Self-inflicted injuries	Self-inflicted injuries are covered.
Sleep Studies	Covered when billed through the enrolled physician's NPI or Outpatient hospital NPI. Medicaid does not enroll sleep study clinics. Unattended sleep studies (95806) are not covered by Medicaid. Please refer to Chapter 19, Hospital, for additional limitations.
Spinal Facet-Joint Interventions	Effective for dates of service on or after May 1, 2024, spinal facet-joint intervention procedure codes CPT 64490-64495 and CPT 64633-64636 will require PA prior to services rendered. Diagnostic Facet-Joint Injection procedures codes CPT 64490, 64491, 64493, & 64494 will be limited to one spinal region (e.g., cervical/thoracic, or lumbar/sacral) per session, with a maximum of two joints per session. In total, each covered spinal region is allowed four sessions per calendar year. Therapeutic Facet-Joint Injection procedure codes CPT 64492 & 64495 are non-covered and describe a third and additional level. Therapeutic Facet-Joint injections are medically necessary only if a recipient is not a candidate for denervation and utilize the same restrictions as diagnostic injections. Facet-Joint Denervation procedure codes CPT 64633, 64634, 64635, & 64636 will be limited to one spinal region (e.g., cervical/thoracic, or lumbar/sacral) per session, with a maximum of four joints per session. In total, each covered spinal region is allowed two sessions per calendar year. All the following requirements must be met for medical necessity to be determined for diagnostic injections: 1) ≥ 3 months of documented back or neck pain that interferes with the recipients 'activities of daily living (ADLs). 2) ≥ 6 weeks of conservative therapy (e.g., physical therapy, home exercise program, activity modification) with documented failure of pain to respond to treatments. 3) ≥ 3 weeks NSAID therapy or NSAID is contraindicated or was not tolerated. 4) Clinical findings suggestive of facet joint syndrome as evidenced by the absence of nerve root compression. 5) Imaging studies suggestive of no other cause for the pain (e.g., disc herniation, radiculitis, discogenic or sacroiliac pain). 6) No prior spinal fusion surgery in the vertebral level being treated. 7) A facet joint denervation intervention is being considered.

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	All the following requirements must be met for medical necessity to be determined for denervation: 1) The recipient has had up to two medically reasonable and necessary diagnostic facet joint injections with each one providing a consistent minimum of 80% pain relief. 2) ≥ 3 months of documented back or neck pain that interferes with the recipients' ADLs. 3) ≥ 6 weeks of conservative therapy (e.g., physical therapy, home exercise program, activity modification) with documented failure of pain to respond to treatments. 4) ≥ 3 weeks NSAID therapy or NSAID is contraindicated or was not tolerated. 5) Clinical findings suggestive of facet joint syndrome and imaging studies suggestive of no other cause for the pain (e.g., fracture, tumor, infection, disc herniation, radiculitis, discogenic or sacroiliac pain, etc.). No prior spinal fusion surgery in the vertebral level being treated.

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Surgery	Cosmetic surgery is covered only when prior approved for medical necessity. Examples of medical necessity include prompt repair of accidental injuries or improvement of the functioning of a malformed body member. Elective surgery is covered when medically necessary. **Multiple and Bilateral Surgical Procedures are governed by the following rules:** Multiple or bilateral surgical procedures that add significant time or complexity and are performed at the same operative session are governed by the following rules: When multiple surgical procedures or bilateral procedures are performed during the same operative session, Medicaid pays for the procedure with the highest allowed amount at 100% and 50% of the allowed amount for each subsequent procedure. This also applies to laser surgical procedures. Procedures are ranked by Medicaid allowed amount (highest to lowest). Additional payments will **not** be made for procedures considered to be mutually exclusive or incidental. Mutually Exclusive procedures are services that cannot reasonably be performed at the same anatomic site or same patient encounter. Certain procedures are commonly carried out as integral parts of a total service and as such do not warrant a separate charge. When incidental procedures (e.g., excision of a previous scar or puncture of an ovarian cyst) are performed during the same operative session, Medicaid reimburses for the major procedure only. CPT defined Add-on codes are considered for coverage when billed with the appropriate primary procedure code. Add-on codes are not subject to rule of 50 percent reduction. Any dressing/compression wrap performed in conjunction with wound debridement is considered part of the debridement services and is not separately covered/billable. Laparotomy is covered when it is the only surgical procedure performed during the operative session or when performed with an unrelated or incidental surgical procedure. Surgeons performing laparoscopic procedures on recipients where a laparoscopic procedure code (PC) has not been established should bill the most descriptive PC with modifier 22 (unusual procedural services) until the new PC is established. Unlisted CPT codes require prior authorization **before** services are rendered. Whenever unusual procedures are performed and there is no exact descriptive CPT code, the Alabama Medicaid Agency requires the most appropriate CPT code be utilized with a modifier 22. Procedure Code 69990 Operating Microscope may be paid separately only when submitted with the following CPT codes: 61304-61546, 61550-61619, 61624-61626, 61640-61711, 62010-62100, 63081-63308, 63704-63710, 64831, 64834-64836, 64840-64858, 64861-64870, 64885-64898, 64905-64907.

Deleted: ~~Multiple surgeries are…laser surgical procedures.~~
Added: Multiple and Bilateral…highest to lowest).

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Deleted: ~~Certain relatively small~~
Added: Minor
Deleted: ~~designated as~~
Added: with a
Added: (000)
Deleted: ~~days may be…post operative care.~~
Added: period includes the…are not payable.

Added: (e.g. LT – Left or RT- Right)
Added: procedure
Deleted: ~~multiple surgeries and~~
Added: procedures performed on different sides or
Deleted: ~~Modifier 57 (Decision…day of surgery.~~
Added: Append modifier 51…of the procedure.

	Minor surgical procedure codes with a "zero" (000) global period includes the preoperative work, the procedure itself, and any related postoperative care provided on the same day as the procedure. Most evaluation and management (E/M) services are not separately billable on the same date as these procedures unless the E/M service is significantly and separately identifiable from the procedure itself. Routine E/M services related to the procedure are not payable. It is necessary to append the appropriate anatomical modifiers (e.g., **LT** – Left or **RT** – Right) to surgical procedure codes to differentiate between procedures performed on different sides or sites. Please refer to Section 28.6.3 Procedure Codes and Modifiers. Append modifier 51 to secondary and additional procedure codes when two or more distinct surgical procedures are performed during the same operative session. Medicaid applies multiple procedure reduction automatically. Modifier 57 (Decision for Surgery) may be used only for major surgical procedures with a 90-day global period. It identifies an Evaluation and Management (E/M) service that results in the initial decision to perform such a procedure. It may be appended to the appropriate E/M code when the decision for surgery is made either the day before or the day of the procedure. NOTE: Surgeons are responsible for digital submission of consent forms and supporting documentation. For hysterectomy and sterilization consent forms please refer to Section 28.6.7 of this chapter regarding submission of consent forms.
Surgery, Breast Reconstruction	Breast reconstruction surgery is reimbursable following a medically necessary mastectomy when performed for the removal of cancer. Breast reconstruction will also be allowed on the non-cancerous contra lateral breast for the purpose of symmetry. Medicaid does **not** reimburse for reconstruction after a prophylactic mastectomy unless evidence of breast cancer is documented in the medical record. All reconstructive procedures require prior authorization. The term "reconstruction" shall include augmentation mammoplasty, reduction mammoplasty, and mastoplexy. Breast reconstruction surgeries are governed by the following rules: • The reconstruction follows a medically necessary mastectomy for the removal of cancer. A pathology report is required. • The recipient is eligible for Medicaid on the date of reconstruction surgery • The recipient elects reconstruction within two years of the mastectomy surgery date • Documentation of therapy completion (chemotherapy and/or radiation treatment), and Operative Report of mastectomy if reconstructive procedure is performed after mastectomy on a different date. If reconstructive procedures are to be performed on the same date as the mastectomy, the physician must send certification that radiation therapy is not planned based on current staging or treatment plan, or must document therapy completion. • For more information regarding prior authorization, please refer to Chapter 4 Obtaining Prior Authorization. For more information related to breast prosthesis, please refer to Chapter 14 Durable Medical Equipment.
Telemedicine	Refer to Chapter 112, Telemedicine Service, for coverage details.

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Therapy	Physician visits for therapy are counted as part of each recipient's benefit limit of 14 physician visits per year. See Rule No. 560-X-6.14 for details about this benefit limit in the *Alabama Medicaid Agency Administrative Code,* Chapter 6. Physical Therapy is not covered when provided in a physician's office. Physical therapy is covered only when prescribed by a physician or non-physician practitioner and provided in a hospital setting. Refer to Chapter 19, Hospital, for more information. For all physical therapy services performed as a result of an EPSDT screening refer to Chapter 37, Therapy, for policy only. Group Therapy is a covered service when a psychiatric diagnosis is present and the therapy is prescribed, performed, and billed by the physician personally. Group Therapy is not covered when performed by a caseworker, social services worker, mental health worker, or any counseling professional other than physician. Group Therapy is included in the physician visit limit of 14 visits per year. Speech Therapy for a speech related diagnosis, such as stroke (CVA) or partial laryngectomy, is a covered benefit when prescribed by and performed by a physician in his office. Speech therapy performed in an inpatient or outpatient hospital setting or in a nursing facility is a covered benefit, but is considered covered as part of the reimbursement made to the facility and should not be billed by the physician. Family Therapy is a covered service when a psychiatric diagnosis is present and the physician providing the service supplies documentation that justifies the medical necessity of the therapy for each family member. Family therapy is not covered unless the patient is present. Family Therapy is not covered when performed by a caseworker, social service worker, mental health worker, or any counseling professional other than a physician. Family Therapy is included in the physician visit limit of 14 visits per year.
Transplants	See Chapter 19, Hospitals, for transplant coverage.
Ventilation Study	Ventilation study is covered if done in physician's office by the physician or under the physician's direct supervision. Documentation in the medical record must contain all of the following: • Graphic record • Total and timed vital capacity • Maximum breathing capacity Always indicate if the studies were performed with or without a bronchodilator.
Newborn Care Services	Well baby coverage is covered only on the initial visit, which must be provided within 8 weeks of birth. When the well-baby checkup is done, the physician should bill procedure code 99461. Diagnosis codes Z00.129, Z00.110, and Z00.111 for ICD-10 are acceptable to bill for routine newborn care/well baby checkup. Only one well-baby checkup can be paid per lifetime, per recipient. Refer to Appendix A, EPSDT, for information on additional preventive services.

NOTE:
For newborn hospital discharge services performed on a subsequent admission date, use code 99238. Please use code 99463 when filing claims for newborns assessed and discharged from the hospital or birthing room on the same date.

Coding Exceptions

Specific codes sets in an audit were identified with an explanation as to why they should be removed or modified in the audit process. Medicaid agrees these codes sets can be billed together as an exception to NCCI and/or CPT policy. As indicated, the multiple surgery rule will be applied.

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Code Sets									***Multiple Surgery***
Procedure code 64450 is allowed with code 54160.									Yes
Tympanostomy 69436 – codes below									Yes
Allowed with 69436		21030	30545	31238	31511	31615	40819	42720	42831
11300	12052	21555	30801	31240	31515	31622	40820	42806	42835
11305	14040	21556	30802	31254	31525	31624	41010	42810	42836
11401	15120	30115	30901	31255	31526	31625	41110	42815	42870
11420	15760	30130	30903	31256	31535	31641	41115	42820	42960
11440	17000	30140	31000	31267	31540	38510	41520	42821	42961
11441	17017	30200	31020	31276	31541	38542	42140	42825	43200
11444	17250	30310	31231	31287	31575	38724	42145	42826	43202
11900	20922	30520	31237	31288	31613	40808	42200	42830	43830

28.2.3 Non-covered Services

Service	***Coverage and Conditions***
Acupuncture	Acupuncture is not covered.
After Office Hours	The following services are not covered: After office hours, services provided in a location other than the physician's office, and office services provided on an emergency basis.
Autopsies	Autopsies are not covered.
Bariatric Procedures	Considered cosmetic unless specific medical criteria are met
Biofeedback	Biofeedback is not covered.
Blood Tests	Blood tests are not covered for marriage licenses.
Casting and Supplies	Some surgical codes are considered an inclusive package of professional services and/or supplies and are not considered separately allowable or reimbursable as the fracture repair or surgical codes is inclusive of these services. An example of this would be a surgical code for a fracture repair which is inclusive of any casting and strapping services or supplies.
Cerumen Removal	CPT Code 69210 is not covered if the ear wax is not impacted and the service does not meet the criteria outlined in section 28.2.2, Covered Services.
Chiropractors	Chiropractic services are not covered, except for QMB recipients and for services referred directly as a result of an EPSDT screening.
Chromosomal Studies	Refer to Chapter 24: Maternity Care for further information.
Dressing and Compression Wrap	Any dressing/compression wrap performed in conjunction with wound debridement is considered part of the debridement services and is not separately covered / billable.
Experimental Treatment or Surgery	Experimental treatment or surgery is not covered.
Filing Fees	Filing Fees are not covered.
Hypnosis	Hypnosis is not covered.
Laetrile Therapy	Laetrile therapy is not covered.

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Service	*Coverage and Conditions*
Mutually Exclusive Procedures	Mutually exclusive procedures are those codes that cannot reasonably be done in the same session and are considered not separately allowable or reimbursable. For example, a vaginal and abdominal hysterectomy on the same date of service.
Oxygen and Compressed Gas	Oxygen therapy is a covered service based on medical necessity and requires prior authorization. Please refer to Chapter 14, DME, for more information.
Pulse Oximetry	Effective January 1, 2009, Non-invasive ear or pulse oximetry services (procedure codes 94760-94762) will no longer be considered separately billable/payable by Medicaid for physician and outpatient services. These procedure codes, per policy dated July 2006, are considered bundled services which are included in Evaluation and Management codes for both physician and outpatient services.
Surgery	When multiple and/or bilateral procedures are billed in conjunction with one another and meet the CPTs definition of "Format of Terminology" (bundled or subset) and/or comprehensive/component (bundled) codes, then the procedure with the highest allowed amount will be paid while the procedure with the lesser amount will not be considered for payment as the procedure is considered an integral part of the covered service. Please refer to Section 28.6.3 Procedure Codes and Modifiers. Incidental surgical procedures are defined as those codes that are commonly carried out as integral parts of a total service and as such do not warrant a separate charge. An example of this would be lysis of adhesions during the same session as an abdominal surgery. Refer to this link, https://www.medicaid.gov/medicaid/program-integrity/national-correct-coding-initiative-medicaid/index.html for more information regarding NCCI.
Post-Surgical Visits	Post-surgical hospital or office visits for conditions directly related to major surgical procedures are covered by the surgical fee. Surgical visits cannot be billed separately the day of surgery or up to 90 days after surgery. Visits by Assistant Surgeon or Surgeons are not covered.
Preventive Medicine	Medicaid does not cover preventive medicine other than those specified elsewhere, including but not limited to, EPSDT screening.
Syntocin	Syntocin is not covered.
Telephone Consultations	Telephone consultations are not covered.
Therapy	Occupational and Recreational Therapies are not covered.

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28.2.4 Limitations on Services

Within each calendar year each recipient is limited to no more than a total of 14 physician visits in offices, hospital outpatient settings, nursing facilities, rural health clinics or Federally Qualified Health Centers.

- Visits not counted under this benefit limit will include (but not limited to): EPSDT, prenatal care, postnatal care, and family planning.
- Recipients receiving cancer treatment during the calendar year are eligible for additional annual physician office visits (*see Cancer Treatment Visits section below).
- Physicians services provided in a hospital outpatient setting that have been certified as an emergency do not count against the physician benefit limit of 14 per calendar year.
- If a patient receives ancillary services in a doctor's office, by the physician or under his/her direct supervision, and the doctor submits a claim only for the ancillary services but not for the office visit, then the services provided will not be counted as a visit.

Office visits are limited to one per day per recipient per provider. For purposes of this limitation, a physician or physician of the same specialty and subspecialty from the same group practice are considered a single provider. Annual office visit benefit limits are 14 office visits per calendar year, although each limit can be exceeded based upon medical necessity.

NOTE:

Medicaid will continue to pay covered ancillary services (injections, lab, x-rays etc.) for recipients after they have exhausted the 14 physician office visit limitation.

Claims for physician's visits which deny due to the recipient exceeding their 14 annual physician office visit limit require manual review and override. The physician must request a redetermination from Gainwell as outlined in Chapter 7, Section 7.2.2. When the redetermination request results in a denial by Gainwell, the provider may request an Administrative Review of the claim as long as the claim is within the timely filing limit by submitting the following along with the claim:

- Copy of previous redetermination correspondence sent to Gainwell
- Copies of all relevant remittances advices or redetermination denial notification
- Completed Medical Utilization Administrative Review Form 402
- An original signed clean Red Drop Ink CMS Form 1500.
- Documentation of medical necessity
- Supporting medical documentation (Op notes, Medical records, supportive reports, etc.)

Send all required documents to the following address:

Alabama Medicaid Agency
Attn: Physician Division
P.O. Box 5624
Montgomery, AL 36103-5624

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For more information regarding the Administrative Review process, refer to Chapter 7, Section 7.2.2, Claims Denials and the Appeals Process.

For further information regarding outpatient maintenance dialysis and ESRD, refer to Chapter 35, Renal Dialysis Facility.

A new patient office visit codes shall not be paid to the same physician or same group practice for a recipient more than once in a three-year period.

Cancer Treatment Visits

Effective January 1, 2023, Alabama Medicaid will increase the annual physician office visit maximum to 32 for Medicaid recipients receiving cancer treatment during the calendar year (January -- December). This increase will be available for each calendar year in which the recipient is receiving cancer treatment and is applicable for all cancers.

To qualify for this increase in annual physician office visits:

1. The claim must include one of the following informational procedure codes to identify the treatment stage:
   - 3300F: American Joint Committee on Cancer (AJCC) state documented and reviewed (ONC)
   - 3301F: Cancer stage documented in medical record as metastatic and reviewed (ONC)
   - S0353: Treatment planning and care coordination management for cancer initial treatment
   - S0354: Treatment planning and care coordination management for cancer established patient with a change in regimen

(Failure to provide one of the required informational procedure codes will cause the recipient to not be eligible for the 32 visits.)

2. A cancer diagnosis, within the current calendar year, must be in the recipient's claims history.

Additionally, medical documentation to support the diagnosis and treatment(s) must be maintained in the recipient's record and provided to the Agency, upon request, in the event of audit.

Prolonged Services Direct Patient Contact in Office or Other Outpatient Setting and Hospital Setting

Effective January 1, 2023, the procedure codes for prolonged services have changed. Procedure code G2212 will be used for the office or other outpatient setting, and procedure code G0316 will be used for the hospital setting.

- **Documentation Requirement**: When prolonged time occurs, the appropriate prolonged services code may be reported. The total time on the date of the encounter spent caring for the patient should be documented in the medical record when it is used as the basis for code selection. Time must be documented clearly in the medical record to indicate the beginning of service time and the end of service time to justify these codes being billed in addition to the office visit.

  1. Physician or other qualified health care professional time includes the following activities, when performed:

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- preparing to see the patient (e.g., review of tests)
- obtaining and/or reviewing separately obtained history
- performing a medically appropriate examination and/or evaluation
- counseling and educating the patient/family/caregiver
- ordering medications, tests, or procedures
- referring and communicating with other health care professionals (when not separately reported)
- documenting clinical information in the electronic or other health record
- independently interpreting results (not separately reported) and communicating results to the patient/family/caregiver
- care coordination (not separately reported)

2. Do not count time spent on the following:
   - the performance of other services that are reported separately
   - travel
   - teaching that is general and not limited to discussion that is required for the management of a specific patient

- When filing claims, procedure codes G2212 or G0316 are assigned only when the code for the primary E&M service has been selected based solely on total time, and only after exceeding by 15 minutes the maximum time that is required to report the highest-level service. For example, when reporting an established patient encounter (99215), code G2212 would not be reported until at least 15 minutes of time beyond 54 minutes has been accumulated (i.e., 69 minutes) on the day of the encounter. **Do not report G2212 or G0316 for any time unit less than 15 minutes. The maximum allowed billable units is 4 at 15-minute intervals.**

  1. G2212 and G0316 must be billed in conjunction with companion procedure codes:
     - G2212 must be filed with 99205 or 99215 for office or other outpatient E&M services.
     - G0316 must be filed with 99223, 99233, or 99236 for hospital inpatient or observation care E&M services.
  2. These services will be subject to post-payment review.

28.2.5 Physician Services to Hospital Inpatients

In addition to the 14 physician visits, Medicaid covers up to 16 inpatient dates of service per physician, per recipient, per calendar year. For purposes of this limitation, each specialty within a group or partnership is considered a single provider. Physician visits are limited to one per day.

When filing claims for recipients enrolled in the ACHN Program, refer to Chapter 40.

Physician hospital visits are limited to one visit per day, per recipient, per provider.

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Physician(s) may bill for inpatient professional interpretation(s), when that interpretation serves as the official and final report documented in the patient's medical record. Professional interpretation may be billed in addition to a hospital visit if the rounding physician also is responsible for the documentation of the final report for the procedure in the patient's medical record. The procedure code must be billed with modifier **26** (Professional Component) and modifier **CG** (Policy criteria applied) appended.

Physician(s) may **not** bill for inpatient professional interpretation(s) in addition to hospital visits if the provider reviews results in the medical record or unofficially interprets medical, laboratory, or radiology tests. Review and interpretation of such tests and results are included in the evaluation and management of the inpatient. Medicaid will cover either one hospital visit or professional interpretation(s) up to the allowed benefit limit for most services.

- Echocardiography (i.e., M-mode, transthoracic, complete and follow up)
- Echocardiography (i.e., 2D, transesophageal)
- Echocardiography (i.e., Doppler pulsed or continuous wave with spectral display, complete and follow up)
- Cardiac Catheterizations
- Comprehensive electrophysiologic evaluations and follow up testing
- Programmed stimulation and pacing
- Intra-operative epicardial and endocardial pacing and mapping
- Intracardiac catheter ablations; intracardiac echocardiography
- Evaluation of cardiovascular function
- Plethysmography, total body and tracing
- Ambulatory blood pressure monitoring
- Cerebrovascular arterial studies, extremity arterial studies, venous studies, and visceral and penile studies
- Circardian respiratory pattern recording (i.e., pediatric pneumogram), infant
- Needle electromyography
- Ischemic limb exercise test
- Assessment of aphasia
- Developmental testing
- Neurobehavioral status exam and neuropsychological testing battery

Professional interpretations for lab and x-ray (CPT code 70000 through 80000 services) in the inpatient setting can only be billed by pathologists and radiologists. The only exception is for professional interpretations by cardiologists for catheterization or arterial studies and for select laboratory procedures by oncologists and hematologists. Professional interpretations/components done by other physicians for services in this procedure code range are included in the hospital visit if one is done. If no hospital visit is made, professional interpretation by physicians other than radiologists, pathologists, oncologists, hematologists, and cardiologists

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should not be billed as these services are covered only for the above-mentioned specialties.

A physician hospital visit and hospital discharge shall not be paid to the same physician on the same day. If both are billed, only the discharge shall be paid.

Professional interpretations performed for an inpatient are counted by dates of service rather than the number of interpretation performed.

An office visit shall not be paid to the same provider or other physicians in the same group practice with the same specialty and subspecialty on the same day as an inpatient visit. If both are billed, then the **first** Procedure Code billed will be paid.

Physician consults are limited to one per day per recipient.

28.2.6 Critical Care (99291 & 99292)

When caring for a critically ill patient, for whom the constant attention of the physician is required, the appropriate critical care procedure code (99291 and 99292) must be billed. Critical care guidelines are defined in the Current Procedural Terminology (CPT) and Provider Manual. Critical care is considered a daily global inclusive of all services directly related to critical care.

Coverage of critical care may total no more than four hours per day.

The actual time period spent in attendance at the patient's bedside or performing duties specifically related to that patient, irrespective of breaks in attendance, must be documented in the patient's medical record.

RESTRICTIONS:

No individual procedures related to critical care may be billed in addition to procedure codes 99291 and 99292, except:

- An EPSDT screening may be billed in lieu of the initial hospital care (P/C 99221, 99222, or 99223). If screening is billed, the initial hospital care cannot be billed.
- Procedure code 99082 (transportation or escort of patient) may also be billed with critical care (99291 and/or 99292 for recipients 25 months of age and older or 99466 and/or 99467 for recipients 24 months of age or less). Only the attending physician may bill this service and critical care. Residents or nurses who escort a patient may not bill either service.

LIMITATIONS:

PROCEDURE CODES NOT BILLABLE IN ADDITION TO CRITICAL CARE (99291 & 99292):

FROM	TO	FROM	TO	FROM	TO	FROM	TO
31500	31500	43752	43757	92265	92275	95925	95937
36000	36440	51100	51100	92280	92287	99090	99091
36468	36479	51701	51702	92920	93299	99170	99199
36510	36510	62270	62270	93303	93352	99460	99463
36555	36555	71010	71020	93561	93562		
36591	36591	82800	82820	93668	93799		
36600	36680	91105	91105	93875	94799		

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- Procedure codes 99291, 99292, 99466 and 99467 may be billed by the physician providing the care of the critically ill or injured patient in place of service 41, Ambulance, if care is personally rendered by the physician providing the care of the critically ill or injured patient.

28.2.7 Pediatric and Neonatal Critical Care

CPT Code	Description	Criteria
99468	Initial Inpatient Neonatal Critical Care, per day for the evaluation and management of a critically ill neonate, 28 days of age or less	Not valid for 29 days or older, can be billed by any physician provider type
99469	Subsequent Inpatient Neonatal Critical Care, per day for the evaluation and management of a critically ill neonate, 28 days of age or less	Not valid for ages 29 days or older, can be billed by any physician provider type
99471	Initial Inpatient Pediatric Critical Care, per day for the evaluation and management of a critically ill infant or young child, 29 days through 24 months of age	Not valid for 28 days or less, can be billed by any physician provider type
99472	Subsequent Inpatient Pediatric Critical Care per day, for the evaluation and management of a critically ill infant or young child, 29 days through 24 months of age	Not valid for ages 28 days or less, can be billed by any physician provider type
99475	Initial inpatient pediatric critical care, per day, for the evaluation and management of a critically ill infant or young child, 2 through 5 years of age	May be billed by any physician provider type for infant or child, 2 through 5 years of age
99476	Subsequent inpatient pediatric critical care, per day, for the evaluation and management of a critically ill infant or young child, 2 through 5 years of age	May be billed by any physician provider type for infant or child, 2 through 5 years of age

The pediatric and neonatal critical care codes (99468-99476) include management, monitoring and treatment of the patient, including respiratory, pharmacological control of the circulatory system, enteral and parenteral nutrition, metabolic and hematologic maintenance, parent/family counseling, case management services and personal direct supervision of the health care team in the performance of their daily activities.

The following criteria should be used as guidelines for the correct reporting of neonatal and pediatric critical care codes for the critically ill neonate/infant. Only one criterion is required to be classified as critically ill.

- Respiratory support by ventilator or CPAP
- Nitric oxide or ECMO
- Prostaglandin, Indotropin of Chronotropic or Insulin infusions

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- NPO with IV fluids
- Acute Dialysis (renal or peritoneal)
- Weight less than 1,250 grams
- Acute respiratory distress in a pediatric admission requiring oxygen therapy with at least daily adjustment and FIO2>35% oxygen.

RESTRICTIONS:

No individual procedures related to critical care may be billed in addition to procedure codes 99468-99476 except:

- Chest tube placement
- Pericardiocentesis or thoracentesis
- Intracranial taps
- Initial hospital care history and physical screen may be billed in conjunction with 99468.
  NOTE: One EPSDT screen for the hospitalization will encompass all diagnoses identified during the hospital stay for referral purposes. The EPSDT screen is considered apart of 99468. Pediatric and neonatal providers should enroll as EPSDT and ESPDT Referred providers and document the referral in the medical record in order to file subsequent claims as EPSDT referred services.
- Standby (99360), resuscitation (99465), or attendance at delivery (99464) may be billed in addition to critical care. Only one of these codes may be billed in addition to neonatal intensive care critical care codes.

LIMITATIONS:

- Pediatric, neonatal critical care codes and intensive (non-critical) low birth weight service codes are reported once per day per recipient.
- Subsequent Hospital Care codes (99231-99233) cannot be billed on the same date of service as neonatal critical care codes (99468-99476)
- Only one unit of critical care can be billed per child per day in the same facility. Medicaid pays the first claim received and denies subsequent claims (first in, first out policy).
- Critical care is considered to be an evaluation and management service. Although usually furnished in a critical or intensive care unit, critical care may be provided in any inpatient health care setting. Services provided which do not meet critical care criteria, should be billed under the appropriate hospital care codes. If a recipient is readmitted to the NICU/ICU, the provider must be the primary physician in order for NICU critical care codes to be billed again.
- Once the patient is no longer considered by the attending physician to be critical, the Subsequent Hospital Care codes (99231-99233) should be billed. Only one unit can be billed per day per physician regardless of specialty. Medicaid pays the first claim received and denies subsequent claims (first in, first out policy).

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- Transfers to the pediatric unit from the NICU cannot be billed using critical care codes. Subsequent hospital care would be billed in these instances.
- Global payments encompass all care and procedures that are included in the rate. Providers may not perform an EPSDT screen and refer to a partner or other physician to do procedures. All procedures that are included in the daily critical care rate, regardless of who performed them, are included in the global critical care code.
- Consultant care rendered to children for which the provider is not the primary attending physician must be billed using consultation codes. Appropriate procedures may be billed in addition to consultations. If, after the consultation the provider assumes total responsibility for care, critical care may be billed using the appropriate critical care codes as defined above. The medical record must clearly indicate that the provider is assuming total responsibility for care of the patient and is the primary attending physician for the patient.

28.2.8 Initial and Continuing Observation or Intensive Care Services

CPT Code	Description	Criteria
99477	Initial hospital care, per day for the evaluation and management of the neonate, 28 days of age or younger, which requires intensive observation, frequent interventions, and other intensive care services.	May only be billed by a neonatologist
99478	Subsequent intensive care, per day, for the evaluation and management of the recovering very low birth weight infant (present body weight less than 1500 grams)	May only be billed by a neonatologist
99479	Subsequent intensive care, per day, for the evaluation and management of the recovering very low birth weight infant (present body weight of 1500-2500 grams)	May only be billed by a neonatologist
99480	Subsequent intensive care, per day, for the evaluation and management of the recovering infant (present body weight of 2501-5000 grams)	May only be billed by a neonatologist

These codes are used to report care subsequent to the day of admission provided by a neonatologist directing the continuing intensive care of the very low birth weight infant who no longer meets the definition of being critically ill. Low birth weight services are reported for neonates less than 2500 grams who do not meet the definition of critical care but continue to require intensive observation and frequent services and intervention only available in an intensive care setting. Services provided to these infants exceed those available in less intensive hospital areas of medical floors. These infants require intensive cardiac and respiratory monitoring, continuous and/or frequent vital signs monitoring, heat maintenance, enteral and/or parenteral nutritional adjustments, laboratory and oxygen

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monitoring and constant observation by the health care team under direct supervision.

Restrictions:

No individual procedures related to critical care may be billed in addition to procedure codes 99478-99480 except:

- Chest tube placement
- Pericardiocentesis or thoracentesis
- Intracranial taps

Limitations:

- Once the patient is no longer considered by the attending physician to be critical, the Subsequent Hospital Care codes (99231-99233) should be billed. Only one unit can be billed per day per physician regardless of specialty. Medicaid pays the first claim received and denies subsequent claims (first in, first out policy).
- Global payments encompass all care and procedures that are included in the rate. Providers may not perform an EPSDT screen and refer to a partner or other physician to do procedures. All procedures that are included in the daily critical care rate, regardless of who performed them, are included in the global critical care code.
- Consultant care rendered to children for which the provider is not the primary attending physician must be billed using consultation codes. Appropriate procedures may be billed in addition to consultations. If, after the consultation the provider assumes total responsibility for care, critical care may be billed using the appropriate critical care codes as per the setting. The medical record must clearly indicate that the provider is assuming total responsibility for care of the patient and is the primary attending physician for the patient.
- Pediatric, neonatal critical care codes and intensive (non-critical) low birth weight services codes are only reported once per day per recipient.

PROCEDURE CODES NOT BILLABLE IN ADDITION TO INITIAL AND CONTINUING OBSERVATION OR INTENSIVE CARE SERVICES

FROM	TO	FROM	TO	FROM	TO	FROM	TO
31500	31579	62263	62368	94002	94004	99218	99220
36000	36830	90470	90471	94010	94772	99231	99239
43752	43761	90760	90781	95831	95857	99251	99275
51000	51010	92081	92081	95880	95882	99291	99292
51100	51102	92551	92551	96101	96125	99431	99435
51600	51798	92950	92950	96360	96379	99460	99463
53670	53675	93000	93352	99090	99091		

28.2.9 End-Stage Renal Disease (ESRD)

Physician services rendered to each outpatient maintenance dialysis patient provided during a full month shall be billed on a monthly capitation basis using the appropriate procedure code by age as outlined in the CPT. Monthly maintenance dialysis payment (i.e., uninterrupted maintenance

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dialysis) is comprehensive and covers most of a physician's services whether a patient dialyzes at home or in an approved ESRD outpatient facility. Dialysis procedures are allowed in addition to the monthly maintenance dialysis payment. In general, the Agency follows Medicare guidelines related to monthly capitation payments for physicians.

Physician services included in the monthly capitation payment for ESRD related services include, but are not limited to:

- Assessment and determination of the need for outpatient chronic dialysis therapy
- Assessment and determination of the type of dialysis access and dialyzing cycle,
- Management of the dialysis visits including outpatient visits for evaluation and management, management during the dialysis, and telephone calls.
- Assessment and determination if a recipient meets preliminary criteria as a renal transplant candidate including discussions with family members
- Assessment for a specified diet and nutritional supplementation for the control of chronic renal failure, including specifying quantity of total protein, sodium, potassium, amount of fluids, types of anemia and appropriate treatments, type of arthropathy or neuropathy and appropriate treatment or referral, estimated ideal dry weight, etc. Assessment for diabetic patient's diet and caloric intake is included also.
- Prescribing the parameters of intradialytic management including anticoagulant, dialysis blood flow rates and temperature, duration and frequency of treatments, etc.

The monthly capitation payment is limited to once per month, per recipient, per provider.

The following services are not covered by the monthly capitation payment (MCP) for the attending dialysis physicians and are reimbursed in accordance with usual and customary charge rules:

- Declotting of shunts
- Covered physician services furnished to hospital inpatients by a physician who elects not to receive the MCP for these services. For example, an attending physician who provides evaluation and management (E & M) services for a renal patient in an inpatient setting may bill appropriate CPT hemodialysis procedures in lieu of certain other E & M services for inpatient visits.

Nonrenal related physician services furnished by the physician providing renal care or by another physician. (These services may not be incidental to services furnished during a dialysis session or office visit necessitated by the renal condition.) For example, physician services rendered to hospitalized inpatient recipients who require dialysis but are not receiving dialysis on that day may use the appropriate procedure code as described in the CPT.

Physician services are allowed for outpatient maintenance dialysis patients not performed as prescribed during a full month or interruptedly. An

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example of interrupted monthly outpatient dialysis maintenance is preceding and/or following the period of hospitalization.

The CPT codes described by age for physicians rendering outpatient dialysis services that are interrupted during a full month should be billed on a per day basis. These codes should be billed for the days of the month in which the outpatient ESRD related services were performed.

Single or repeated physician assessments are allowed for hemodialysis or dialysis procedures other than hemodialysis. These services are comprehensive and include assessment and management related to the patient's renal dialysis. Please utilize the most descriptive and appropriate CPT dialysis procedure when billing for single or repeated physician evaluation(s).

Dialysis training is a covered service when billed by an approved ESRD facility.

Refer to Chapter 35, Renal Dialysis Facility, for further details.

Parenteral Nutrition

The Alabama Medicaid Agency may reimburse for total parenteral nutritional (TPN) solutions through the pharmacy program if the recipient meets certain requirements as listed below. TPN solutions include those used for hyperalimentation, intradialytic parenteral nutrition (IDPN), and intraperitoneal nutrition (IPN). Requirements must be met and clearly documented in the medical record for coverage of all TPN. All services rendered are subject to post payment review.

Statement of Medical Necessity

The ordering physician will be responsible for writing a statement of medical necessity. This statement shall certify that the patient cannot be maintained on oral or enteral feedings and that due to severe pathology of the alimentary tract either hyperalimentation or IDPN/IPN must be given for 100% of nutritional needs. The original signed statement of medical necessity must be kept in the patient's medical record. This certification statement must be written or stamped on the prescription or reproduced on a form accompanying the prescription. The statement must be signed and dated by the certifying physician at the time of the initial order and updated yearly in accordance with Medicaid billing practice.

Hyperalimentation

Medicaid covers hyperalimentation for recipients who meet certain requirements of medical necessity and documentation in the medical record is sufficient based on the following:

- Documentation in the medical record must validate the patient suffers from a permanently impaired gastrointestinal tract and that there is insufficient absorption of nutrients to maintain adequate strength and weight. The following are considered conditions which could cause insufficient absorption:
    1. Crohn's disease

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2. Obstruction secondary to stricture or neoplasm of the esophagus or stomach

3. Loss of ability to swallow due to central nervous system disorder, where the risk of aspiration is great

4. Short bowel syndrome secondary to massive small bowel resection

5. Malabsorption due to enterocolic, enterovesical or enterocutaneous fistulas (TPN temporary until the repair of the fistula)

6. Motility disorder (pseudo-obstruction)

7. Prolonged paralytic ileus following a major surgical procedure or multiple injuries

8. Newborn infants with catastrophic gastrointestinal anomalies such as tracheoesophageal fistulas, gastroschisis, omphalocele or massive intestinal atresia

9. Infants and young children who fail to thrive due to systemic disease or secondary to insufficiency associated with short bowel syndrome, malabsorption or chronic idiopathic diarrhea.

- Medical record documentation must include supporting evidence that the patient cannot be maintained on oral or enteral feedings and that due to severe pathology of the alimentary tract, hyperalimentation must be given in order to meet 100% of the patient's nutritional needs.
- Physical signs, symptoms and test results indicating severe pathology of the alimentary tract must be clearly documented in the medical record. This would include BUN, serum albumin, and phosphorus. Medical records must document inability to maintain weight during a trial of at least four weeks of enteral feeding.

Intradialytic Parenteral Nutrition (IDPN) and Intraperitoneal Nutrition (IPN)

IDPN and IPN involves infusing hyperalimentation fluids as part of dialysis, through the vascular shunt or intraperitoneally to normalize the amounts of albumin, glucose, and other nutrients in the blood stream to decrease morbidity and mortality associated with protein calorie malnutrition. IDPN and IPN solutions are considered **not covered** for the recipient with a functioning gastrointestinal tract whose need for parenteral nutrition is only due to the following:

- If IDPN or IPN is offered as an addition to regularly scheduled infusions of TPN
- If the recipient would not qualify as a candidate for TPN
- A swallowing disorder
- A temporary defect in gastric emptying such as a metabolic or electrolyte disorder
- A psychological disorder, such as depression, impairing food intake
- A metabolic disorder inducing anorexia, such as cancer
- A physical disorder impairing food intake, such as dyspnea or severe pulmonary or cardiac disease
- A side effect of medication
- Renal failure and/or dialysis

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The following requirements must be met in order to bill for IDPN or IPN solutions:

- Documentation in the medical record must validate the patient suffers from a permanently impaired gastrointestinal tract and that there is insufficient absorption of nutrients to maintain adequate strength and weight.
- Documentation must include that the patient cannot be maintained on oral or enteral feedings and that due to severe pathology of the alimentary tract, IDPN or IPN must be given in order to meet 100% of the patient's nutritional needs.
- Infusions must be vital to the nutritional status of the recipient and not supplemental to a deficient diet or deficiencies caused by dialysis.
- Physical signs, symptoms and test results indicating severe pathology of the alimentary tract must be clearly documented in the medical record. This would include creatinine (predialysis), serum albumin (predialysis), a low or declining serum cholesterol level, and phosphorus. Medical records must document inability to maintain weight during a trial of at least four weeks of enteral feeding.

Restrictions

A few solutions used in TPN preparation are considered payable as part of the composite rate for dialysis and should not be billed separately by the pharmacist; these are as follows:

- Glucose
- Dextrose
- Trace Elements
- Multivitamins

28.2.10 Anesthesiology

Administration of anesthesia is a covered service when administered by or directed by a duly licensed physician for a medical procedure that is covered by Medicaid. Medical direction by an anesthesiologist of more than four Certified Registered Nurse Anesthetists (CRNAs) or Anesthesiology Assistants (AAs) concurrently will not be covered.

Administration of anesthesia by an AA is a covered service when the AA has met the qualifications and standards set forth by the Alabama Board of Medical Examiners. The AA must enroll with a NPI to bill the Alabama Medicaid Program. Refer to Chapter 38, Anesthesiology, for more information.

28.2.11 Obstetrical and Related Services

Refer to Chapter 24: Maternity Care for further detailed information.

28.2.12 Vaccines For Children (VFC)

The Department of Public Health provides vaccines at no charge to Medicaid providers enrolled in the Vaccines For Children (VFC) Program as recommended by the Advisory Committee on Immunization.

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Medicaid reimburses administration fees for vaccines provided free of charge through the VFC Program. The rate for the administration fee is $8.00; it is not the rate on the pricing file. Please refer to Appendix A, Well Child Check-Up, Section A.6.1 (Fees) for information regarding the use of designated VFC codes for billing immunization administration fee(s).

A VFC provider may or may not choose to become an enrolled Medicaid provider. Enrollment as a VFC provider or a Medicaid provider is independent of each other.

Refer to Appendix A, EPSDT, for procedure codes for VFC.

28.2.13 Lab Services

Collection of laboratory specimens may be billed only when sending specimens to another site for analysis if the other site is not owned, operated, or financially associated with the site in which the specimen was collected. The collection fee may not be billed if the lab work is done at the same site where the specimen was collected or in a lab owned, operated, or financially associated with the site in which the specimen was collected. Providers will not be paid for and should not submit claims for laboratory work done for them by independent laboratories or by hospital laboratories.

Providers may submit claims for laboratory work done by them in their own offices or own laboratory facilities. Providers who send specimens to independent laboratories for analysis may bill a collection fee. This fee shall not be paid to any provider who has not actually extracted the specimen from the patient.

Lab Tests Performed in Physician's Offices

When performing laboratory tests in the physician's office:

1. The Physician must be CLIA certified to perform the test,
2. The Physician must have the appropriate equipment to perform the test, and
3. The Physician's office bills for the tests performed but not the collection fee.

When specimens are sent to an outside lab:

1. The Physician's office should not bill the laboratory code, and
2. The Physician's office may bill a collection fee with a "90" modifier for blood specimens.

EXAMPLE:

Lead Levels

Procedure Code 83655 (Lead) should only be billed when the office has the equipment to perform the test. When collecting a specimen only and then sending the blood sample to an outside lab for analysis, you must bill Procedure Code 36415 with modifier 90. The utilization of procedure code 36415-90 will enable you to receive a collection and handling fee for the specimen obtained.

Procedure code 36415-90 should not be billed when lab procedures are performed in the office. The appropriate lab procedure code(s) must be billed when actually performing the lab test. Again, the correct equipment must be utilized to perform the test. These services are subject to post-

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payment review. Medical record documentation must support the performance and medical necessity of the laboratory test.

NOTE:

Providers should use procedure code 36415-90 for routine venipuncture collection, 36416-90 for collection of capillary blood specimen (e.g., finger, heel, ear stick) and Q0091-90 for collection of Pap smear specimen.

Repeat Lab Procedures

Modifier 91 may be utilized to denote a repeat clinical laboratory test performed on the same date of service for the same recipient. Providers should use modifier 91 instead of modifier 76 for repeat lab procedures.

NOTE:

A physician CANNOT bill the following pathology/laboratory procedure codes; however the above collection fee can be billed, if applicable:

82775 Galactose – 1 – phosphate uridyl transferase; quantitative
83498 Hydroxyprogesterone, 17 – d
84030 Phenylalanine (PKU) blood
84437 Thyroxine; total requiring elution (e.g., neonatal)

28.2.14 Supply Code

The procedure code 99070 is utilized by physicians to bill for supplies and materials over and above those usually included with the office visit. Examples of supplies and materials over and beyond usual supplies include elastic wraps, disposable tubing for bronchial dilating equipment or post-operative dressing changes when no office visit is allowable.

28.3 Prior Authorization and Referral Requirements

Medical care and services that require prior authorization for in-state providers will continue to require prior authorization for out-of-state providers, e.g., organ transplants and select surgical procedures. Please refer to Chapter 4, Obtaining Prior Authorization for more information.

For information regarding Prior Authorization for MRI's, CT scans, CTA's, MRA's, and PET scans, refer to chapter 22, Independent Radiology.

Unlisted services and procedure codes are not covered by the Alabama Medicaid Agency, with the exception of Medicare crossover claims and rare instances when approval is granted prior to service provision after the agency has determined that the service is covered and that no other procedure code exists for reimbursement.

NOTE:

It is the responsibility of the physician to obtain prior authorization for any outpatient surgical procedure to be performed in an outpatient hospital.

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28.4 Signature Requirements

Signature Requirement for Referrals: Effective May 16, 2012, for hard copy referrals, the printed, typed, or stamped name of the primary care physician with an original signature of the physician or designee is required. **Stamped or copied signatures will not be accepted.** For electronic referrals, provider certification is made via standardized electronic signature protocol.

Services that require a physician's order must be written according to practice guidelines and state/federal law and must include the date and signature of the provider, the service(s) ordered and the recipient's name. The EPSDT referral form may be considered the physician's order as long as these guidelines are met. Refer to the individual provider manual chapters for detailed description of what must be included in an order.

All entries in the medical record must be legible and complete, and must be authenticated and dated promptly by the person (identified by name and discipline) who is responsible for ordering, providing, or evaluating the service furnished. The author of each entry must be identified and must authenticate his/her entry. Authentication may include handwritten or electronic signatures, or written initials, **Stamped or copied signatures will not be accepted.**

28.5 Cost Sharing (Copayment)

The copayment amount for office visit* including crossovers is:

$3.90 for procedure codes reimbursed $50.01 or more
$2.60 for procedure codes reimbursed between $25.01 - $50.00
$1.30 for procedure codes reimbursed between $10.01 - $25.00
$0.65 for procedure codes reimbursed $10.00 or less

* The following CPT codes are considered office visits and the copayment is based on Medicaid's allowed amount (fee schedule) for each procedure:

90847, 90849, 90853, 90865, 92002, 92004, 92012, 92014, 99202, 99203, 99204, 99205, 99211, 99212, 99213, 99214, 99215, 99241, 99242, 99243, 99244, 99245, 99281, 99282, 99283, 99284, 99285, 99341, 99342, 99343, 99344, 99345, 99347, 99348, 99349, 99350

If one of these CPT codes is applicable for your practice, then copay applies.

Copayment does not apply to services provided for pregnant women, nursing facility residents, recipients less than 18 years of age, emergencies, and family planning. Native American Indians that present an "active user letter" issued by Indian Health Services (IHS) will be exempt from the Medicaid required copayment.

The provider may not deny services to any eligible Medicaid recipient because of the recipient's inability to pay the cost-sharing (copayment) amount imposed.

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28.6 Completing the Claim Form

To enhance the effectiveness and efficiency of Medicaid processing, providers should bill Medicaid claims electronically.

Physicians who bill Medicaid claims electronically receive the following benefits:

- Quicker claim processing turnaround
- Immediate claim correction
- Enhanced online adjustment functions
- Improved access to eligibility information

Refer to Appendix B, Electronic Media Claims Guidelines, for more information about electronic filing.

NOTE:

When filing a claim on paper, a CMS-1500 claim form is required. Medicare-related claims must be filed using the Medical Medicaid/Medicare-related Claim Form.

This section describes program-specific claims information. Refer to Chapter 5, Filing Claims, for general claims filing information and instructions.

28.6.1 Time Limit for Filing Claims

Medicaid requires all claims for physicians to be filed within one year of the date of service. Refer to Section 5.1.5, Filing Limits, for more information regarding timely filing limits and exceptions.

28.6.2 Diagnosis Codes

The *International Classification of Diseases - 10th Revision - Clinical Modification* (ICD-10-CM) manual lists required diagnosis codes. These manuals may be obtained by contacting the American Medical Association, AMA Plaza 330 North Wabash Ave, Suite 39300 Chicago, IL 60611-5885, or 1-800-621-8335.

Additional Diagnosis Codes Submission

Physicians, nurse practitioners, physician assistants, and EPSDT screening providers are able to submit up to 12 additional diagnosis codes per date of service by submitting an additional claim with CPT 99499 (unlisted/unspecified) as a claim detail. Claims for this code should not include other procedure codes. Medicaid will pay $0.00 on claims for CPT 99499. Federally qualified health centers (FQHCs) and rural health clinics (RHCs) should bill CPT 99499 as a fee-for-service claim. Prospective payment rates will not be reimbursed for claims for this code.

NOTE:

ICD-10 codes should be used for claims submitted with dates of service on or after 10/01/2015.

ICD-10 diagnosis codes must be listed to the highest number of digits possible (3, 4, or 5 digits). Do not use decimal points in the diagnosis code field.

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28.6.3 Procedure Codes and Modifiers

Medicaid uses the Healthcare Common Procedure Coding System (HCPCS). HCPCS is composed of the following:

- American Medical Association's Current Procedural Terminology (CPT)
- Nationally assigned codes developed for Medicare
- Locally assigned codes issued by Medicaid. Effective for dates of service on or after 01/01/2004, use national codes.

Filing Claims with Modifiers

Appropriate use of CPT and HCPCS modifiers is required to differentiate between sites and procedures. It is necessary to append the appropriate anatomical modifiers to surgical codes to differentiate between multiple surgeries and sites.

Appropriate Use of Modifiers

Please refer to this CMS link for more information regarding NCCI edits: https://www.medicaid.gov/medicaid/program-integrity/national-correct-coding-initiative-medicaid/index.html

Modifier 24 (Unrelated Evaluation and Management Service by the Same Physician During a Postoperative Period)

The Physician may need to indicate that an evaluation and management service was performed during a postoperative period for a reason(s) unrelated to the original procedure. This circumstance may be reported by adding modifier 24 to the appropriate level of E&M service. From a coding perspective, modifier 24 is appropriate when a physician provides a surgical service related to one problem and, during the postoperative period or follow-up care for the surgery, provides and E&M service unrelated to the problem requiring the surgery.

Modifier 25 (Significant, Separately Identifiable Evaluation and Management Service by the Same Physician on the Same Day of the Procedure or Other Service)

It may be necessary to indicate that on the day a procedure or service identified by a CPT code was performed, the patient's condition required a significant, separately identifiable E&M service above and beyond the other service provided or beyond the usual preoperative and postoperative care associated with the procedure that was performed. A significant, separately identifiable E&M service is defined or substantiated by documentation that satisfies the relevant criteria for the respective E&M service to be reported.

Modifiers 59, XE, XP, XS, and XU (Distinct Procedural Services)

Under certain circumstances, the physician may need to indicate that a procedure or service was distinct or independent from other non-E&M services performed on the same day. Modifiers 59, XE, XP, XS, and XU are used to identify procedures/services, other than E&M services, that are not normally reported together, but are appropriate under the circumstances. Documentation must support a different session, different procedure or surgery, different site or organ system, separate incision/excision, separate lesion, or separate injury (or area of injuries in

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extensive injuries) not ordinarily encountered or performed on the same day by the same individual. This may represent a different session or patient encounter, different procedure or surgery, different site or organ system, separate incision/excision, separate lesion, or separate injury (or area of injury in extensive injuries) not ordinarily encountered or performed on the same day by the same physician. However, when another already established modifier is appropriate, it should be used rather than modifier 59. Only if no more descriptive modifier is available, and the use of modifier 59 best explains the circumstances, should modifier 59 be used.

Modifiers XE, XP, XS, and XU are effective for dates of service beginning January 1, 2015 and thereafter. These modifiers were developed to provide greater reporting specificity in situations where modifier 59 was previously reported and may be utilized in lieu of modifier 59 whenever possible. (Modifier 59 should only be utilized if no other more specific modifier is appropriate.) The modifiers are defined as follows:

- **XE – Separate encounter**: A service that is distinct because it occurred during a separate encounter.
- **XP – Separate practitioner**: A service that is distinct because it was performed by a different practitioner.
- **XS – Separate structure**: A service that is distinct because it was performed on a separate organ/structure.
- **XU – Unusual non-overlapping service**: The use of a service that is distinct because it does not overlap usual components of the main service.

Modifier 76 (Repeat Procedure by Same Physician)

The physician may need to indicate that a procedure or service was repeated subsequent to the original procedure or service. This circumstance may be reported by adding the modifier 76 to the repeated procedure/service. From a coding perspective, modifier 76 is intended to describe the same procedure or service repeated, rather than the same procedure being performed at multiple sites.

Modifier 78 (Return to the Operating Room for a Related Procedure During the Postoperative Period)

It may be necessary to indicate that another procedure was performed during the postoperative period of the initial procedure. When the subsequent procedure is related to the first, and requires the use of the operating room, it may be reported by adding the modifier 78 to the related procedure. Modifier 78 is not used for procedures that indicate in the code descriptor “subsequent, related or redo.”

Modifiers 80, 81, 82 and AS (Assistant-at-Surgery Modifiers)

An assistant-at-surgery serves as an additional pair hands for the operating surgeon. Assistants-at-surgery do not carry primary responsibility for or “perform distinct parts” of the surgical procedure. Assistant-at-surgery coverage is limited to fully qualified physicians and non-physician practitioner (i.e., PAs, CRNP, etc.) if it is within the scope of their licenses.

- **Modifier 80** – Assistant surgeon

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- **Modifier 81** – Minimum assistant surgeon
- **Modifier 82** – Assistant surgeon (when qualified resident surgeon not available)
- **Modifier AS** – Physician assistant, nurse practitioner, or clinical nurse specialist for assistant at surgery.

Modifier Q5 (Service Performed by a Substitute Physician under a Reciprocal Billing Agreement)

Under certain circumstances, the physician may need to indicate that a service was provided by a substitute physician. Modifier Q5 is reported when the regular physician arranges for a substitute physician to furnish services on an occasional reciprocal basis. Modifier Q5 should be appended after the procedure code to indicate that the service was provided by a substitute physician under a reciprocal arrangement. When appending modifier Q5, the regular provider is certifying that the services are covered services furnished by the substitute physician. The regular physician should keep a record on file of each service provided by the substitute physician and make this record available to Medicaid upon request.

Modifier Q6 (Service Furnished by a Locum Tenens Physician)

Under certain circumstances, the physician may need to indicate that a service was provided by a locum tenens physician. A locum tenens physician generally has no practice of his/her own; he/she usually moves from area to area as needed. The regular physician generally pays the substitute physician a fixed per diem amount or other fee-for-time compensation, with the locum tenens physician having a status of an independent contractor rather than of an employee. Modifier Q6 should be appended after the procedure code to indicate that the service was provided by a locum tenens physician. The regular physician should keep a record on file of each service provided by the locum tenens physician and make this record available to Medicaid upon request.

Bilateral Procedures

Effective for dates of adjudication October 1, 2006 and thereafter the procedure for billing bilateral procedures changed. In the past, (through September 30, 2006), providers were instructed to bill for bilateral procedures on one line with modifier 50. The reimbursement was adjusted to 150% of Medicaid's fee schedule.

Effective for dates of adjudication October 1, 2006 and thereafter, the new procedure is as follows:

- Bill the appropriate procedure code on 2 separate lines with RT and LT modifier, or other appropriate anatomical modifier,
- Modifier 50 will be used for informational purposes only and is no longer a pricing modifier.
- The payment will be 100% of Medicaid fee schedule for first line and 50% for second line.
- Claims will be subject to multiple surgery payment adjustments for multiple procedures.

Example:

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Line 1: 27558 RT
27558 LT; 50 (Optional use of modifier 50)

Alabama Medicaid's Fiscal Agent utilizes Optum Insight Tool to determine whether a 50 modifier or RT and LT modifier should be allowed with the procedure code billed. When an inappropriate procedure code is billed with modifier 50 or RT and/or LT modifier, the claim will deny.

NOTE:
When Medicaid payment occurs for a procedure code billed inappropriately with modifier 50, AND/OR RT (right) AND/OR LT (left), the claim will be subject to a system adjustment in payment, post payment review, and recoupment.

Procedure Codes

The CPT manual lists most procedure codes required by Medicaid. This manual may be obtained by contacting the Order Department, American Medical Association, 515 North State Street, Chicago, IL 60610-9986. The (837) Professional electronic claim and the paper claim have been modified to accept up to four Procedure Code Modifiers.

NOTE:
Unlisted procedure codes are not covered by the Agency unless the provider requested and received approval for a prior authorization before the service is rendered. The Agency will deny all requests for payment of unlisted codes after the fact.

Physician-Employed Physician Assistants (PA) and Certified Registered Nurse Practitioners (CRNP)

CRNP and PA payment will be made only for CPT codes identified in Chapter 21, physician administered drugs, and laboratory services, (must be CLIA certified). EPSDT screenings will be covered only if the provider is enrolled in that program. Refer to Appendix A, EPSDT, for EPSDT program requirements.

The Physician's Assistant or CRNP can make physician required inpatient visits to nursing facilities. However, physician required inpatient visits to hospitals or other institutional settings cannot be made by a PA or CRNP. CRNP and PA services have been expanded. Please refer to Chapter 21 for additional information.

Global Surgical Packages

Effective for dates of adjudication 10/1/06 and thereafter, Medicaid will adopt Medicare's RVU file designation for global surgical days. In the past and through date of adjudication September 30, 2006, Medicaid has used a 62 day post op period after major surgeries.

Effective for dates of adjudication 10/1/06 and thereafter, Medicaid will use a zero, 10 day, and 90 day post op period for routine surgical care. Routine post-surgical care in the hospital or office setting for conditions directly related to surgical procedures is covered by the surgical fee. Depending on post-operative period, post-surgical visits cannot be billed separately the day of, or up to 90 days after surgery.

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For conditions unrelated to the surgical procedure bill the appropriate (E&M) procedure code with a 24 modifier appended. The diagnosis must support use of the modifier 24.

Claims for these services will be subject to post payment review.

Refer to this Medicare RVU file:http://www.cms.gov/apps/physician-fee-schedule/overview.aspx for global surgical procedure codes Zero, 10 and 90 day(s) post-operative period.

Professional and Technical Components

Some procedure codes in the 70000, 80000, 90000, and G series are a combination of a professional component and a technical component. Therefore, these codes may be billed one of three different ways; (1) as a global, (2) as a professional component, or (3) as a technical component. NOTE: Not all providers are allowed to bill any or all of the three ways to bill. Specific coverage questions should be addressed to the Provider Assistance Center.

- **Global**, the provider must own the equipment, pay the technician, review the results, and provide a written report of the findings. The procedure code is billed with no modifiers. The Global component should be billed only for the following place of service locations:
  - 11 (Office)
  - 81 (Independent Laboratory)
- **Professional component**, the provider does not own or operate the equipment. The provider reviews the results, and provides a written report of the findings. The Radiological professional component is billed by adding modifier 26 to the procedure code, and should be billed only for the following place of service locations:
  - 21 (inpatient hospital)
  - 22 (outpatient hospital)
  - 23 (emergency room - hospital)
  - 51 (inpatient psychiatric facility)
  - 61 (comprehensive inpatient rehab facility)
  - 62 (comprehensive outpatient rehab facility)
  - 65 (end-stage renal disease facility)
  - 81 (Independent Laboratory)
- **Technical component**, the provider must own the equipment, but does not review and document the results. The technical component charges are the facility's charges and are not billed separately by physicians. The technical component is billed by adding modifier TC to the procedure code. The technical component can only be billed by facilities.

28.6.4 Billing for ACHN Referred Service

Please refer to Chapter 40 for information regarding the ACHN Program.

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28.6.5 Place of Service Codes

The following place of service codes apply when filing claims for physicians:

POS	Description
02	Telemedicine Services
11	Office
12	Home
21	Inpatient Hospital
22	Outpatient Hospital
23	Emergency Room - Hospital
24	Ambulatory Surgical Center
25	Birthing Center
26	Military Treatment Facility
31	Skilled Nursing Facility or Nursing Facility
32	Nursing Facility
33	Custodial Care Facility
34	Hospice
41	Ambulance - Land
42	Ambulance - Air or water
51	Inpatient Psychiatric Facility
52	Psy. Fac. Partial Hospitalization
53	Community Mental Health Center
54	Intermediate Care Fac./ Individuals with Intellectual Disabilities
55	Residential Substance Abuse Treatment Facility
56	Psychiatric Residential Treatment Center
61	Comprehensive Inpatient Rehabilitation Facility
62	Comprehensive Outpatient Rehabilitation Facility
65	End Stage Renal Disease Treatment Facility
71	State or Local Public Health Clinic
72	Rural Health Clinic

28.6.6 Required Attachments

To enhance the effectiveness and efficiency of Medicaid processing, your attachments should be limited to the following circumstances:

- Claims With Third Party Denials

NOTE:

When an attachment is required, a hard copy CMS-1500 claim form must be submitted.

Refer to Section 5.8, Required Attachments, for more information on attachments.

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28.6.7 Consent Forms Required Before Payments Can Be Made

NOTE:

Gainwell will NOT pay any claims to ANY provider until a correctly completed original of the appropriate form is on file at Gainwell. Please note, **only the surgeon** should submit consent forms to Gainwell. All other providers should not request and or submit copies of the consent form. Multiple copies slow down the consent form review and claims payment process.

Abortions

In accordance with federal law, abortions are covered only (1) if the pregnancy is the result of an act of rape or incest; or (2) where a woman suffers from a physical disorder, physical injury, or physical illness, including a life-endangering physical condition caused by or arising from the pregnancy itself, that would, as certified by a physician, place the woman in danger of death unless an abortion is performed.

On July 1, 2016, Alabama Medicaid implemented fillable Portable Document Format (PDF) versions of the Abortion Consent forms that enable faster processing of provider submitted information. Providers **must** use these fillable consent forms with the digital submission of Consent Forms and supporting documentation. Any form received that is not in a fillable format will be returned to the provider.

Effective October 26, 2016, providers will be able to upload or fax their fillable Consent Forms (Abortion) and supporting documentation for review and processing via the Forms menu of the Alabama Medicaid Interactive Web Portal. A new form will allow providers the ability to upload Consent Forms and supporting documents in PDF format or create a fax barcode cover sheet from the Web Portal. Providers may submit additional documentation via fax at a later time and have that documentation combined with original document through the use of the same barcode cover sheet.

Providers must submit the completed form and supporting documentation via Provider Web Portal upload or fax to Gainwell at: (334) 215-7416.

NOTE:

Abortion Consent forms and supporting documentation will be accepted in paper format via mail or fax until November 26, 2016. After that date, consent forms and supporting documentation submitted to Gainwell on paper will be returned to the provider.

Please refer to Chapter 5, Filing Claims, for instructions on using the digital submission of Consent Forms and supporting documentation.

In the case of abortions performed secondary to pregnancies resulting from rape or incest, the documentation required is a letter from the recipient or provider certifying that the pregnancy resulted from rape or incest.

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- A second copy of the consent form or certification letter must be placed in the recipient's medical record.

All claims relating to abortions must have the above-specified documentation on file at Gainwell prior to payment.

This documentation is not required when a recipient presents with a spontaneous abortion.

If the recipient does not qualify for payment by Medicaid and elects to have the abortion, providers may bill the recipient for the abortion as a non-covered service.

The fillable consent forms are available under the Resource tab on the Alabama Medicaid website at: www.medicaid.alabama.gov.

Sterilization

Gainwell must have on file the Medicaid-approved sterilization form. Refer to Appendix C, Family Planning, for more information. See Appendix E, Medicaid Forms for a copy of this form, or visit the Medicaid website at: www.medicaid.alabama.gov. for an electronic fillable version of this form.

Effective October 27, 2016, providers will be able to upload or fax their fillable Consent Forms (Sterilization) and supporting documentation for review and processing via the Forms menu of the Alabama Medicaid Interactive Web Portal. A new form will allow providers the ability to upload Consent Forms and supporting documents in PDF format or create a fax barcode cover sheet from the Web Portal. Providers may submit additional documentation via fax at a later time and have that documentation combined with original document through the use of the same barcode cover sheet. The provider must submit a copy of the recipient's signed sterilization consent form to Gainwell via Provider Web Portal upload or fax. Fax form and supporting documentation to Gainwell at: (334) 215-7416.

Refer to Appendix C for detailed instructions.

Sterilization by Hysterectomy

Payment is not available for a hysterectomy if:

1. **It was performed solely for the purpose of rendering an individual permanently incapable of reproducing**
2. **If there was more than one purpose to the procedure, it would not have been performed but for the purpose of rendering the individual permanently incapable of reproducing**

NOTE:

Sterilization performed for the sole purpose of rendering a person permanently incapable of reproducing is not available to persons under twenty-one (21) years of age under the Medicaid Program.

Refer to Appendix E, Medicaid Forms, for a sample of the sterilization form.

Hysterectomy

Hysterectomy procedures performed for the sole purpose of rendering an individual incapable of reproducing are not covered under Medicaid.

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Hysterectomies done as a medical necessity as treatment of disease can be paid for by the Medicaid funds under the physician's program.

Effective October 1, 2025, the hysterectomy consent form is revised to include language that certifies the procedure is being performed for the treatment of disease and not for the prevention of disease or prophylaxis. With the revision to the consent form, the hysterectomy review process is also updated. If the hysterectomy is performed for treatment of disease, the provider may submit by uploading or faxing the signed consent form by using the Alabama Medicaid Interactive Web Portal so the claim can be processed. There is no need to submit medical records for hysterectomies that are medically necessary. However, if the hysterectomy was performed due to unusual circumstances (e.g., already sterile, life-threatening situation, or performed under a period or retroactive Medicaid eligibility), the provider must submit medical records and consent form by uploading or faxing the documents to the Web Portal. An administrative review will be performed by Medicaid before the claim is processed. All claims are subject to post-payment review for medical necessity and compliance with Alabama Medicaid policies and procedures.

NOTE:

The **doctor's explanation** to the patient that the operation will make her sterile and the **doctor's and recipient's signature** must precede the operation except in the case of medical emergency. If a field is missing, contains invalid information or indicates the recipient/representative or physician signed after the date of surgery, Gainwell will return the consent form to the provider to correct invalid information.

Gainwell must have on file a Medicaid-approved Hysterectomy Consent Form.

Alabama Medicaid implemented fillable Portable Document Format (PDF) versions of the Hysterectomy Consent form that enables faster processing of provider submitted information.

Providers must use their fillable Consent Forms (Hysterectomy) via the Forms menu of the Alabama Medicaid Interactive Web Portal. If the hysterectomy was medically necessary, no additional medical records are needed. However, if the hysterectomy was performed due to unusual circumstances, the provider must upload the consent form and medical records via the Forms menu of the Alabama Medicaid Interactive Web Portal. The Web Portal will allow providers the ability to upload Consent Forms and supporting documents in PDF format or create a fax barcode cover sheet. Providers may submit additional documentation via fax at a later time and have that documentation combined with original document through the use of the same barcode cover sheet.

The fillable consent forms are available under the Resource Tab on the Alabama Medicaid website in the section labeled Consent Forms at: www.medicaid.alabama.gov.

Providers must submit the completed form and supporting documentation via Provider Web Portal upload or fax to Gainwell at: (334) 215-7416.

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NOTE:

The old Hysterectomy Consent form may be used until January 1, 2026. After that date, only new consent forms will be accepted.

Please refer to Chapter 5, Filing Claims, for instructions on using the digital submission of Consent Forms and supporting documentation.

Exceptions That Do Not Require Prior Completion of the Consent Form

In the following situations, the consent form is required and section III and IV of the consent form must be completed.

1. The physician who performed the hysterectomy certifies in writing that the patient was already sterile when the hysterectomy was performed; the cause of sterility must be stated in this written statement. Refer to Section IV on the consent form.
2. The physician who performed the hysterectomy certifies in writing that the hysterectomy was performed under a life-threatening emergency situation in which prior acknowledgement was not possible. This written statement must include a description of the nature of the emergency. Refer to Section IV on the consent form.
3. The hysterectomy was performed during a period of retroactive Medicaid eligibility, and the physician who performed the hysterectomy submits, in lieu of the consent form, a written statement certifying that the individual was informed before the operation that the hysterectomy would make her sterile. Refer to Section IV on the consent form.

NOTE:

Medicaid payment cannot be made for any claims for services provided in connection with an abortion, a sterilization procedure or a hysterectomy for medical reasons unless an approved consent form is on file. Please be aware consent for sterilization is different from consent for hysterectomy. See Appendix E, Medicaid Forms, for examples of each

28.7 For More Information

This section contains a cross-reference to other relevant chapters in the manual.

Resource	Where to Find It
CMS-1500 Claim Filing Instructions	Chapter 5
Medical Medicaid/Medicare-related Claim Filing Instructions	Chapter 5
Medical Necessity/Medically Necessary Care	Chapter 7
Anesthesiology	Chapter 38
EPSDT	Appendix A
Electronic Media Claims (EMC) Submission Guidelines	Appendix B
Family Planning	Appendix C
Alabama Medicaid Injectable Drug List	Medicaid website
AVRS Quick Reference Guide	Appendix L
Alabama Medicaid Contact Information	Appendix N
CRNP and PA Services	Chapter 21

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